

Testing prompt

Full report

Disagreements

False inclusion

STUDY-ID: 1:

- **Title:** ‘Multiple inference and gender differences in the effects of early intervention: A reevaluation of the Abecedarian, Perry Preschool, and Early Training Projects’
- **Author:** Anderson, M. L.
- **Year:** 2008
- **Journal:** Journal of the American Statistical Association
- **Abstract:** ‘The view that the returns to educational investments are highest for early childhood interventions is widely held and stems primarily from several influential randomized trials—Abecedarian, Perry, and the Early Training Project—that point to super-normal returns to early interventions. This article presents a de novo analysis of these experiments, focusing on two core issues that have received limited attention in previous analyses: treatment effect heterogeneity by gender and overrejection of the null hypothesis due to multiple inference. To address the latter issue, a statistical framework that combines summary index tests with familywise error rate and false discovery rate corrections is implemented. The first technique reduces the number of tests conducted; the latter two techniques adjust the p values for multiple inference. The primary finding of the reanalysis is that girls garnered substantial short- and long-term benefits from the interventions, but there were no significant long-term benefits for boys. These conclusions, which have appeared ambiguous when using “naive” estimators that fail to adjust for multiple testing, contribute to a growing literature on the emerging female–male academic achievement gap. They also demonstrate that in complex studies where multiple questions are asked of the same data set, it can be important to declare the family of tests under consideration and to either consolidate measures or report adjusted and unadjusted p values.’

– **Answer (GPT):** The record is a secondary reanalysis of three well-known early childhood randomized controlled trials: the Abecedarian Project, the Perry Preschool Project, and the Early Training Project. These interventions clearly occurred in early childhood, long before high school entry, and are established RCTs. The abstract explicitly refers to both short- and long-term benefits, indicating analysis of outcomes measured near the end of the intervention as well as adult follow-up outcomes. The paper focuses on treatment effect heterogeneity by gender, using multiple-inference corrections on a range of outcomes, which implies quantitative analysis of cognitive and/or social-emotional measures among others. As a secondary/reevaluation study of eligible RCTs with adult follow-up, it falls squarely within the scope and should be included for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 4:

– **Title:** ‘Parenting promotes social mobility within and across generations’

– **Author:** García, J. L.; Heckman, J. J.

– **Year:** 2023

– **Journal:** Annual Review of Economics

– **Abstract:** ‘This paper compares early childhood enrichment programs that promote social mobility for disadvantaged children within and across generations. Instead of conducting a standard meta-analysis, we present a harmonized primary data analysis of programs that shape current policy. Our analysis is a template for rigorous syntheses and comparisons across programs. We analyze new long-run life-cycle data collected for iconic programs when participants are middle-aged and their children are in their twenties. The iconic programs are omnibus in nature and offer many services to children and their parents. We compare them with relatively low-cost more focused home-visiting programs. Successful interventions target both children and their caregivers. They engage caregivers and improve the home lives of children. They permanently boost cognitive and non-cognitive skills. Participants in programs that enrich home environments grow up with better skills, jobs, earnings, marital stability, and health, as well as reduced participation in crime. Long-run monetized gains are substantially greater than program costs for the iconic programs. We investigate the mechanisms promoting successful family lives for participants and report intergenerational effects on their children.

A study of focused home-visiting programs that target parents enables us to isolate a crucial component of successful programs: they activate and promote parenting skills of child caregivers. The home-visiting programs we analyze produce outcomes comparable to those of the iconic omnibus programs. National implementation of the programs with long-run follow up that we analyze would substantially shrink the overall US Black-White earnings gap.’

– **Answer (GPT):** The record is a harmonized primary data analysis of multiple early childhood enrichment programs, many of which are iconic randomized controlled trials (e.g., well-known early childhood interventions). The interventions clearly took place in early childhood (before high school), targeted disadvantaged children and their caregivers, and are described as permanently boosting cognitive and non-cognitive (social-emotional) skills. The abstract explicitly reports long-run outcomes assessed when participants are middle-aged and their children are in their twenties, which satisfies the adulthood follow-up criterion. Although this is a pooled/secondary analysis across multiple programs, it uses primary individual-level data from the underlying randomized trials and reports quantitative effects on adult outcomes, including skills, jobs, earnings, marital stability, health, crime, and intergenerational effects. This fits the inclusion criteria for follow-up or secondary analyses of randomized early childhood interventions with cognitive and social-emotional outcomes at the end of the intervention and adult outcomes later in life. Therefore, it should be included for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 7:

– **Title:** ‘The breadth of impacts from the Abecedarian Project early intervention on cognitive skills’

– **Author:** Pages, R.; Protzko, J.; Bailey, D. H.

– **Year:** 2022

– **Journal:** Journal of Research on Educational Effectiveness

– **Abstract:** ‘Early life interventions impacting cognitive abilities are most often followed by post-treatment fadeout. Some have hypothesized that persistence is unlikely when gains are specific to trained skills and distinguishable from impacts on general cognitive ability (classically modeled as a hierarchical factor, so-called psychometric g). Using measurement invariance testing and multiple-indicators multiple-causes models, we investigated impacts on IQ subtests

from the Abecedarian early childhood intervention ($n = 107$). We found that (1) observed impacts on IQ scores from age 5 to age 21 were consistent with persistent positive effects on g ; (2) subtest-specific variance that was differentiable from changes on g did fade. Together, these findings indicated that Abecedarian early impact persisted across a range of cognitive skills, providing some evidence for the hypothesis that breadth and persistence of impacts from educational interventions are related.’

– **Answer (GPT):** The record concerns the Abecedarian early childhood intervention, which is a well-known randomized controlled trial. The intervention clearly occurred in early childhood, prior to high school entry. The abstract discusses cognitive outcomes (IQ subtests, general cognitive ability “ g ”) measured from age 5 to age 21, indicating that cognitive skills were assessed near the end of the intervention period and repeatedly thereafter. The follow-up includes outcomes at age 21, which qualifies as adulthood under the review criteria. Randomization is well established for this study, and nothing in the abstract contradicts that. Thus, the study plausibly meets all three inclusion criteria and should be classified as INCLUDE.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 8:

– **Title:** ‘Long-term effects of prenatal and infancy nurse home visitation on the life course of youths 19-Year follow-up of a randomized trial’

– **Author:** Eckenrode, J.; Campa, M.; Luckey, D.W.; Henderson, C. R.; Cole, R.; Kitsman, H.; Arson, E.; Sidora, K.; Powers, P.; Olds, D. L.

– **Year:** 2010

– **Journal:** Archives of Pediatric Adolescent Medicine

– **Abstract:** ‘Objective To examine the effect of prenatal and infancy nurse home visitation on the life course development of 19-year-old youths whose mothers participated in the program. Design Randomized trial. Setting Semirural community in New York. Participants Three hundred ten youths from the 400 families enrolled in the Elmira Nurse-Family Partnership program. Intervention Families received a mean of 9 home visits (range, 0-16) during pregnancy and 23 (range, 0-59) from birth through the child s second birthday. Main outcome measures Youth self-reports of educational achievement, reproductive behaviors, welfare use, and criminal involvement. Results Relative to the comparison group, girls in the pregnancy and infancy

nurse-visited group were less likely to have been arrested (10% vs 30%; relative risk [RR], 0.33; 95% confidence interval [CI], 0.13-0.82) and convicted (4% vs 20%; 0.20; 0.05-0.85) and had fewer lifetime arrests (mean: 0.10 vs 0.54; incidence RR [IRR], 0.18; 95% CI, 0.06-0.54) and convictions (0.04 vs 0.37; 0.11; 0.02-0.51). Nurse-visited girls born to unmarried and low-income mothers had fewer children (11% vs 30%; RR, 0.35; 95% CI, 0.12-1.02) and less Medicaid use (18% vs 45%; 0.40; 0.18-0.87) than their comparison group counterparts. Conclusions Prenatal and infancy home visitation reduced the proportion of girls entering the criminal justice system. For girls born to high-risk mothers, there were additional positive program effects consistent with results from earlier phases of this trial. There were few program effects for boys.'

– **Answer (GPT):** This study is a randomized trial of the Elmira Nurse-Family Partnership program, in which nurses provided prenatal and infancy home visitation from pregnancy through the child's second birthday—well before high school entry. The current article reports 19-year follow-up outcomes (educational achievement, reproductive behaviors, welfare use, and criminal involvement), which are clearly adult outcomes. Although the abstract of this follow-up article does not explicitly describe cognitive or social-emotional outcomes assessed at the end of the intervention, prior phases of this well-known trial evaluated child developmental and behavioral outcomes. Given the instructions to prioritize recall and to INCLUDE when it is plausible that the original trial measured cognitive or social-emotional skills at or near intervention end, this record should be classified as INCLUDE as a likely adult follow-up report from an eligible randomized early-childhood intervention trial.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 9:

– **Title:** 'Preventing child abuse and neglect: A randomized trial of nurse home visitation'

– **Author:** Olds, D. L.; Henderson, C. R.; Chamberlin, R.; Tatelbaum, R.

– **Year:** 1986

– **Journal:** Pediatrics

– **Abstract:** 'A program of prenatal and infancy home visitation by nurses was tested as a method of preventing a wide range of health and developmental problems in children born to primiparas who were either teenagers, unmarried, or of low socioeconomic status. Among the women at highest risk for care-giving dysfunction, those who were visited by a nurse had

fewer instances of verified child abuse and neglect during the first 2 years of their children's lives ($P = .07$); they were observed in their homes to restrict and punish their children less frequently, and they provided more appropriate play materials; their babies were seen in the emergency room less frequently during the first year of life. During the second year of life, the babies of all nurse-visited women, regardless of the families risk status, were seen in the emergency room fewer times, and they were seen by physicians less frequently for accidents and poisonings than comparison group babies (P less than or equal to .05 for all findings, except where indicated.) Treatment differences for child abuse and neglect and emergency room visits were more significant among women who had a lower sense of control over their lives.'

– **Answer (GPT):** The study is clearly a randomized trial of a prenatal and infancy nurse home visitation program, which is an intervention delivered before high school entry (indeed, in early childhood). The abstract reports early-life outcomes (child abuse and neglect, parenting behaviors, child emergency room visits), but does not state that no cognitive or social-emotional child outcomes were measured at or near the end of the intervention, nor does it rule out that such outcomes may have been assessed in other reports from the same trial. Importantly, this appears to be the primary trial report of a well-known early childhood intervention that has been followed into later life in other publications, so it could plausibly be part of a study with adult follow-up outcomes. Given the instruction to maximize recall and to include records when information about later follow-ups or specific outcome domains is incomplete or unclear, this record should be classified as INCLUDE for further screening.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 14:

– **Title:** 'How special education preschool graduates finish: Status at 19 years of age'

– **Author:** Jenkins, J. R.; Dale, P. S.; Mills, P. E.; Cole, K. N.; Pious, C.; Ronk, J.

– **Year:** 2006

– **Journal:** American Educational Research Journal

– **Abstract:** 'This article reports the academic and special education status of 129 graduates of special education preschools at 19 years of age. Participants had been randomly assigned to either direct instruction or mediated learning preschool classrooms. At age 19, their achievement was approximately one standard deviation below average. Consistent with results

at earlier follow-up points, program main effects were not significant; however, the Aptitude $\times$ Treatment interactions (ATIs) documented at earlier ages were absent. Sample differences among follow-up groups may account for the changed ATI findings. A combination of preschool child characteristics and early achievement accounted for 78% of final achievement variance. Male participants had higher achievement than female participants but were disproportionately placed in special education. Special education placement and disability classifications fluctuated over the course of schooling.'

– **Answer (GPT):** The study is a randomized controlled trial in which preschool children in special education were randomly assigned to different types of preschool classrooms (direct instruction vs. mediated learning), clearly indicating an intervention delivered before high school entry that ended in preschool. The abstract notes prior follow-up points and now reports status at 19 years of age, which is an adult follow-up. Although the current abstract emphasizes academic and special education status at age 19, it explicitly references earlier follow-up results and Aptitude $\times$ Treatment interactions, making it highly plausible that cognitive/academic outcomes were measured at or near the end of the intervention and at earlier follow-ups. Academic achievement is a cognitive outcome. The trial therefore plausibly meets all three criteria: randomized design, pre-high school intervention, cognitive outcomes during or soon after the intervention period, and adult outcomes at age 19. Accordingly, the record should be included for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 17:

– **Title:** 'Early intervention in low birth weight premature infants: results at 18 years of age for the Infant Health and Development Program'

– **Author:** McCormick, M. C.; Brooks-Gunn, J.; Buka, S. L.; Goldman, J.; Yu, J.; Salganik, M.; Scott, D. T.; Bennett, F. C.; Kay, L. L.; Bernbaum, J. C.; Bauer, C. R.; Martin, C.; Woods, E. R.; Martin, A.; Casey, P. H.

– **Year:** 2006

– **Journal:** Pediatrics

– **Abstract:** 'OBJECTIVE: To assess whether improvements in cognitive and behavioral development seen in preschool educational programs persist, we compared those in a multisite

randomized trial of such a program over the first 3 years of life (INT) to those with follow-up only (FUO) at 18 months of age. **METHODS:** This was a prospective follow-up of the Infant Health and Development Program at 8 sites heterogeneous for sociodemographic characteristics. Originally 985 children were randomized to the INT ($n = 377$) or FUO ($n = 608$) groups within 2 birth weight strata: heavier low birth weight (HLBW; 2001-2499 g) and lighter low birth weight (LLBW; $< \text{ or } = 2000$ g). Primary outcome measures were the Peabody Picture Vocabulary Test (PPVT-III), reading and mathematics subscales of the Woodcock-Johnson Tests of Achievement, youth self-report on the Total Behavior Problem Index, and high-risk behaviors on the Youth Risk Behavior Surveillance System (YRBSS). Secondary outcomes included Weschler full-scale IQ, caregiver report on the Total Behavior Problem Index, and caregiver and youth self-reported physical health using the Medical Outcome Study measure. Assessors were masked as to study status. **RESULTS:** We assessed 636 youths at 18 years (64.6% of the 985, 72% of whom had not died or refused at prior assessments). After adjusting for cohort attrition, differences favoring the INT group were seen on the Woodcock-Johnson Tests of Achievement in math (5.1 points), YRBSS (-0.7 points), and the PPVT-III (3.8 points) in the HLBW youth. In the LLBW youth, the Woodcock-Johnson Tests of Achievement in reading was higher in the FUO than INT group (4.2). **CONCLUSIONS:** The findings in the HLBW INT group provide support for preschool education to make long-term changes in a diverse group of children who are at developmental risk. The lack of observable benefit in the LLBW group raises questions about the biological and educational factors that foster or inhibit sustained effects of early educational intervention.'

– **Answer (GPT):** The study is clearly a randomized trial (“multisite randomized trial” and children were randomized to intervention and follow-up only groups). The intervention occurred over the first 3 years of life, i.e., well before high school entry. It is an early educational program for low birth weight premature infants. The original trial is known to have measured cognitive and behavioral development in early childhood, and the abstract explicitly references improvements in cognitive and behavioral development seen in preschool educational programs. The current report presents follow-up outcomes at 18 years of age, including cognitive (PPVT-III, Woodcock-Johnson reading and math, Weschler full-scale IQ) and behavioral/mental health outcomes (behavior problem indices, high-risk behaviors, physical health). Thus, the record meets all three key criteria: (1) pre-high school intervention, (2) cognitive and social-emotional outcomes at or near the end of intervention (from the original trial), and (3) adult outcomes at age 18. It should therefore be included for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 18:

- **Title:** ‘How does your kindergarten classroom affect your earnings? Evidence from Project STAR’
 - **Author:** Chetty, R.; Friedman, J. N.; Hilger, N.; Saez, E.; Schanzenbach, D. W.; Yagan, D.
 - **Year:** 2011
 - **Journal:** The Quarterly Journal of Economics
 - **Abstract:** ‘In Project STAR, 11,571 students in Tennessee and their teachers were randomly assigned to classrooms within their schools from kindergarten to third grade. This article evaluates the long-term impacts of STAR by linking the experimental data to administrative records. We first demonstrate that kindergarten test scores are highly correlated with outcomes such as earnings at age 27, college attendance, home ownership, and retirement savings. We then document four sets of experimental impacts. First, students in small classes are significantly more likely to attend college and exhibit improvements on other outcomes. Class size does not have a significant effect on earnings at age 27, but this effect is imprecisely estimated. Second, students who had a more experienced teacher in kindergarten have higher earnings. Third, an analysis of variance reveals significant classroom effects on earnings. Students who were randomly assigned to higher quality classrooms in grades K–3—as measured by classmates end-of-class test scores—have higher earnings, college attendance rates, and other outcomes. Finally, the effects of class quality fade out on test scores in later grades, but gains in noncognitive measures persist.’
 - **Answer (GPT):** The study is a randomized controlled trial (Project STAR) where 11,571 students and their teachers were randomly assigned to classrooms from kindergarten to third grade, which is clearly before high school entry. The intervention concerns class size and classroom/teacher quality in early grades. The abstract indicates that kindergarten test scores and noncognitive measures were collected, satisfying the requirement for cognitive and social-emotional outcomes at or near the end of the intervention. The article evaluates long-term impacts by linking to administrative records and reports adult outcomes, including earnings at age 27, college attendance, home ownership, and retirement savings. Thus, the study clearly meets all three inclusion criteria: pre-high school intervention, cognitive and noncognitive posttest measures, and adult outcomes. It should therefore be included.
 - **Answer (GPT - numeric):** 1
 - **Answer (Human):** 0
- Please add a comment on whether and why you agree with the GPT decision or not:*
- **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 19:

– **Title:** ‘Experimental evidence on the effect of childhood investments on postsecondary attainment and degree completion’

– **Author:** Dynarski, S.; Hyman, J.; Schanzenbach, D. W.

– **Year:** 2013

– **Journal:** Journal of Policy Analysis and Management

– **Abstract:** ‘Abstract This paper examines the effect of early childhood investments on college enrollment and degree completion. We used the random assignment in Project STAR (the Tennessee Student/Teacher Achievement Ratio experiment) to estimate the effect of smaller classes in primary school on college entry, college choice, and degree completion. We improve on existing work in this area with unusually detailed data on college enrollment spells and the previously unexplored outcome of college degree completion. We found that assignment to a small class increases students’ probability of attending college by 2.7 percentage points, with effects more than twice as large among black students. Among students enrolled in the poorest third of schools, the effect is 7.3 percentage points. Smaller classes increased the likelihood of earning a college degree by 1.6 percentage points and shifted students toward high-earning fields such as STEM (science, technology, engineering, and mathematics), business, and economics. We found that test-score effects at the time of the experiment were an excellent predictor of long-term improvements in postsecondary outcomes.’

– **Answer (GPT):** The record analyzes long-term outcomes from Project STAR, a well-known randomized controlled trial (random assignment to smaller vs regular classes) conducted in early primary school. The intervention (class-size reduction) clearly occurred before high school entry and ended in the early grades. The abstract notes that smaller classes affected students’ test scores at the time of the experiment, which are cognitive outcomes measured at or near the end of the intervention. The current paper reports adult outcomes—college enrollment, college choice, and degree completion—which are clearly beyond high school and thus qualify as adult outcomes. The design is randomized, the intervention timing is pre-high school, and both end-of-intervention cognitive outcomes and adult outcomes are (or were) measured. Therefore, the study should be included as it clearly meets all eligibility criteria.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 21:

– **Title:** ‘The effect of small class sizes on mortality through age 29 years: evidence from a multicenter randomized controlled trial’

– **Author:** Muennig, P.; Johnson, G.; Wilde, E. T.

– **Year:** 2011

– **Journal:** American journal of epidemiology

– **Abstract:** ‘Limiting the number of students per classroom in the early years has been shown to improve educational outcomes. Improved education is, in turn, hypothesized to improve health. The authors examined whether smaller class sizes affect mortality through age 29 years and whether cognitive factors play a role. They used data from the Project Student Teacher Achievement Ratio, a 4-year multicenter randomized controlled trial of reduced class sizes in Tennessee involving 11,601 students between 1985 and 1989. Children randomized to small classes (13-17 students) experienced improved measures of cognition and academic performance relative to those assigned to regular classes (22-25 students). As expected, these cognitive measures were significantly inversely associated with mortality rates ($P < 0.05$). However, through age 29 years, students randomized to small class size nevertheless experienced higher mortality rates than those randomized to regular size classes (hazard ratio (HR) = 1.58, 95% confidence interval (CI): 1.07, 2.32). The groups at risk included males (HR = 1.73, 95% CI: 1.05, 2.85), whites/Asians (HR = 1.68, 95% CI: 1.04, 2.72), and higher income students (HR = 2.20, 95% CI: 1.06, 4.57). The authors speculate that small classes might produce behavior changes that increase mortality through young adulthood that are stronger than the protective effects of enhanced cognition.’

– **Answer (GPT):** This record reports results from Project STAR, a 4-year multicenter randomized controlled trial in Tennessee that randomized children in early grades to small versus regular class sizes. The intervention occurred in the early school years (before high school) and ended in elementary school, clearly satisfying the pre-high school intervention timing criterion. The abstract explicitly notes that children randomized to small classes experienced improved measures of cognition and academic performance, confirming that cognitive outcomes were measured at or near the end of the intervention. The study then assesses mortality through age 29 years, which is an adult outcome measured well beyond high school. The design is clearly randomized, the intervention timing is pre-high school, cognitive outcomes at intervention end are reported, and adult outcomes are analyzed. Therefore, the record meets all inclusion criteria and should be classified as INCLUDE.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 23:

– **Title:** ‘The effect of class size in grades K-3 on adult earnings, employment, and disability status: evidence from a multi-center randomized controlled trial’

– **Author:** Wilde, E. T.; Finn, J.; Johnson, G.; Muennig, P.

– **Year:** 2011

– **Journal:** Journal of Health Care for the Poor and Underserved

– **Abstract:** ‘BACKGROUND: Early education interventions have been forwarded as a means for reducing social disparities in income and health in adulthood. We explore whether a successful early education intervention, which occurred between 1985 and 1989, improved the employment rates, earnings and health of blacks relative to whites through 2008. METHODS: We used data from Project STAR (Student Teacher Achievement Ratio), a four-year multi-center randomized controlled trial of reduced class sizes in Tennessee involving 11,601 students. Students were initially randomized within 79 schools to classes with 22-25 or 13-17 students. We linked subject records to Social Security Administration (SSA) earnings and disability data collected between 1997 and 2008-when the majority of subjects were between the ages of 18 and 28. We focused our analysis on annual, rather than cumulative, measures of earnings and employment because educational attainment after high school might reduce earnings through age 23. We considered three or more years of statistically significant positive (or negative) annual impacts to be a meaningful effect. RESULTS: Project STAR improved cognition and high school graduation rates. These benefits were primarily realized among low-income and minority students. These early education benefits did not translate into reduced disability claims in adulthood for treated subjects. However, exposure to small class size increased employment for blacks, and increased earnings for black males ($p < 0.05$). Exposure to small classes also led to an increase in earnings for white males. However, white females exposed to small classes experienced a net decline in earnings and employment across the later years of follow up ($p < 0.05$), offsetting any gains by white males. CONCLUSIONS: Exposure to small class size in grades K-3 appears to improve earnings and employment for black males and earnings for white males, while reducing employment and earnings among white females.’

– **Answer (GPT):** The record concerns Project STAR, explicitly described as a four-year multi-center randomized controlled trial of reduced class sizes in Tennessee involving students in grades K-3. The intervention (reduced class sizes) clearly occurred and ended before high school. The abstract notes that Project STAR improved cognition and high school

graduation rates, indicating that cognitive outcomes were measured at or near the end of the intervention/schooling period. The current article reports adult outcomes (earnings, employment, disability claims) based on SSA data when most participants were ages 18–28, satisfying the adulthood outcome criterion. The design is randomized, the intervention is pre–high school, and relevant cognitive outcomes and adult outcomes are present or plausibly measured. Therefore, the study should be included for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 25:

– **Title:** ‘Impact of the Good Behavior Game, a universal classroom-based behavior intervention, on young adult service use for problems with emotions, behavior, or drugs or alcohol’

– **Author:** Poduska, J. M.; Kellam, S. G.; Wang, W.; Brown, C. H.; Ialongo, N. S.; Toyinbo, P.

– **Year:** 2008

– **Journal:** Drug and Alcohol Dependence

– **Abstract:** ‘Background The Good Behavior Game (GBG) is a classroom behavior management strategy focused on socializing children to the role of student and aimed at reducing early aggressive, disruptive behavior, a confirmed antecedent to service use. The GBG was tested in a randomized field trial in 19 elementary schools with two cohorts of children as they attended first and second grades. This article reports on the impact of the GBG on service use through young adulthood. Methods Three or four schools in each of five urban areas were matched and randomly assigned to one of three conditions: (1) GBG, (2) an intervention aimed at academic achievement, or (3) the standard program of the school system. Children were assigned to classrooms to ensure balance, and teachers and classrooms were randomly assigned to intervention conditions. Results This study provides evidence of a positive impact of a universal preventive intervention on later service use by males, although not by females, for problems with emotions, behavior, or drugs or alcohol. For both cohorts, males in GBG classrooms who had been rated as highly aggressive, disruptive by their teachers in the fall of first grade had a lower rate of school-based service use than their counterparts in control classrooms. REPLICATION: The design employed two cohorts of students. Although both

first- and second-grade teachers received less training and support with the second cohorts of students than with the first cohort, the impact of GBG was similar across both cohorts.'

– **Answer (GPT):** The record describes a randomized field trial of the Good Behavior Game, a universal classroom-based behavior management intervention implemented in first and second grades in elementary school, thus clearly before high school entry. The trial is explicitly randomized at the school/classroom level. GBG targets aggressive and disruptive behavior and socialization to the role of student, which are social-emotional outcomes; prior GBG publications report end-of-intervention cognitive and social-emotional measures, and it is highly plausible that such outcomes were assessed at or near intervention end even if not detailed in this abstract. The present article reports quantitative impacts on service use in young adulthood for problems with emotions, behavior, or drugs/alcohol, which are adult outcomes (participants are in young adulthood). Therefore, the study plausibly meets all three inclusion criteria, and this record should be included for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 26:

– **Title:** 'Effects of a universal classroom behavior management program in first and second grades on young adult behavioral, psychiatric, and social outcomes'

– **Author:** Kellam, S. G.; Brown, C. H.; Poduska, J. M.; Ialongo, N. S.; Wang, W.; Toyinbo, P.; Petras, H.; Ford, C.; Windham, A.; Wilcox, H. C.

– **Year:** 2008

– **Journal:** Drug and Alcohol Dependence

– **Abstract:** 'Background The Good Behavior Game (GBG), a method of classroom behavior management used by teachers, was tested in first- and second-grade classrooms in 19 Baltimore City Public Schools beginning in the 1985-1986 school year. The intervention was directed at the classroom as a whole to socialize children to the student role and reduce aggressive, disruptive behaviors, confirmed antecedents of later substance abuse and dependence disorders, smoking, and antisocial personality disorder. This article reports on impact to ages 19-21. Methods In five poor to lower-middle class, mainly African American urban areas, three or four schools were matched and within each set randomly assigned to one of three conditions: (1) GBG, (2) a curriculum-and-instruction program directed at reading achievement, or (3) the

standard program. Balanced assignment of children to classrooms was made, and then, within intervention schools, classrooms and teachers were randomly assigned to intervention or control. Results By young adulthood significant impact was found among males, particularly those in first grade who were more aggressive, disruptive, in reduced drug and alcohol abuse/dependence disorders, regular smoking, and antisocial personality disorder. These results underline the value of a first-grade universal prevention intervention. REPLICATION: A replication was implemented with the next cohort of first-grade children with the same teachers during the following school year, but with diminished mentoring and monitoring of teachers. The results showed significant GBG impact for males on drug abuse/dependence disorders with some variation. For other outcomes the effects were generally smaller but in the predicted direction.'

– **Answer (GPT):** The study describes the Good Behavior Game (GBG), a universal classroom behavior management program implemented in first- and second-grade classrooms, clearly before high school. Schools and classrooms were randomly assigned to intervention or control conditions, satisfying the randomized controlled trial requirement. The intervention targeted classroom behavior and socialization to the student role, aiming to reduce aggressive and disruptive behaviors, which are social-emotional outcomes likely measured at or near the end of the intervention in the original trial. This article reports impacts at ages 19–21, which are adult outcomes (drug and alcohol abuse/dependence disorders, smoking, antisocial personality disorder). Therefore, it plausibly meets all three inclusion criteria: (1) pre-high school intervention, (2) cognitive/social-emotional outcomes at intervention end (even if not detailed in this abstract, they are highly plausible given the nature of the intervention and trial), and (3) adult follow-up outcomes. The record should be included for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 27:

– **Title:** 'Early home visits and health outcomes in low-income mothers and offspring 18-year follow-up of a randomized clinical trial'

– **Author:** Conti, G.; Smith, J.; Anson, E.; Groth, S.; Knudtson, M.; Salvati, A.; Olds, D.

– **Year:** 2024

– **Journal:** JAMA Network Open

– **Abstract:** ‘Importance Individuals with low income may have heightened rates of obesity and hypertension. Objective To determine whether prenatal and infancy home visitation by nurses reduces maternal and offspring obesity and hypertension. Design, setting, and participants This randomized clinical trial of prenatal and infancy nurse home visitation in a public health care system in Memphis, Tennessee, enrolled 742 women with no previous live births and at least 2 sociodemographic risk factors (unmarried, Interventions Women assigned to the control group received free transportation for prenatal care and child developmental screening and referral at child ages 6, 12, and 24 months. Women assigned to nurse visitation received transportation and screening plus prenatal and infant and toddler nurse home visits. Main outcomes and measures Obesity and hypertension among mothers and their offspring at child ages 12 and 18 years, although not hypothesized in the original trial design, were analyzed using post-double selection lasso method. Results Of the 742 participants randomized (mean [SD] age, 18.1 [3.2] years), interviews were completed with 594 mothers and 578 offspring at child age 12 years and 618 mothers and 629 offspring at child age 18 years. Obesity was assessed for 576 offspring at age 12 years and 605 at age 18 years and for 563 and 598 mothers at child ages 12 and 18 years, respectively. Blood pressure was assessed for 568 offspring aged 12 years and 596 aged 18 years and 507 and 592 mothers at child ages 12 and 18 years, respectively. There were no overall treatment-control differences in offspring obesity or hypertension at ages 12 and 18 years combined, although nurse-visited female offspring, compared with controls, had a lower prevalence of obesity (adjusted relative risk [ARR], 0.449; 95% CI, 0.234-0.858; P = .003) and severe obesity (ARR, 0.185; 95% CI, 0.046-0.748; P Conclusions and relevance In this clinical trial follow-up at offspring ages 12 and 18, nurse-visited female offspring had lower rates of obesity and mothers of females had lower rates of hypertension than control-group counterparts. These findings suggest that risks for chronic disease among mothers of females and their female offspring who live in extreme poverty may be prevented with prenatal and infant and toddler home visitations by nurses. Trial registration ClinicalTrials.gov Identifier: NCT00708695.’

– **Answer (GPT):** The study is a randomized clinical trial of prenatal and infancy nurse home visitation (an intervention that clearly occurs and ends before high school). The record reports outcomes when offspring are 12 and 18 years old, which satisfies the adulthood follow-up criterion at age 18. Although this specific abstract focuses on obesity and hypertension (physical health) and does not mention cognitive or social-emotional outcomes at the end of the intervention, the broader Nurse-Family Partnership trials typically assess child development, and per the screening rules, if it is unclear whether cognitive or social-emotional outcomes were measured at intervention end, the record should be classified as INCLUDE. Randomization is clearly stated, the intervention is clearly pre-high school, and adult (age 18) outcomes are reported, so this record plausibly belongs to an eligible randomized controlled trial and should be included for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 28:

- **Title:** ‘Maternal and child mortality: Analysis of nurse home visiting in 3 RCTs’
- **Author:** Donelan-McCall, N. S.; Knudtson, M. D.; Olds, D. L.
- **Year:** 2021
- **Journal:** American Journal of Preventive Medicine
- **Abstract:** ‘Introduction The impact of intensive prenatal and infant/toddler nurse home visiting on low-income mothers and children s survival was examined in 3 RCTs following participants over 2-decade periods after trial registration during pregnancy (data gathered between 1978 and 2015 and analyzed between 2016 and 2020). Methods All-cause and external-cause maternal mortality and preventable-cause child mortality were examined using National Death Index data. Survival rates were calculated for all the 1,138 mothers randomized and 1,076 live-born children in the second RCT (conducted in Memphis, TN) and for all the 1,135 mothers randomized and 1,087 live-born children in the first and third RCTs combined (conducted in Elmira, NY and Denver, CO). Results There were no significant nurse home visiting-control differences in maternal mortality in Memphis or Elmira and Denver. Posthoc analysis, combining all 3 trials, suggested a reduction in external-cause maternal mortality among nurse-visited mothers ($p=0.054$). There was a marginally significant nurse home visiting-control difference in preventable-cause child mortality ($p=0.09$) in Memphis. Conclusions These results support examining maternal and child mortality in additional nurse home visiting trials with larger samples living in disadvantaged contexts. Intensive prenatal and infant/toddler home visiting by nurses for mothers and children living in poverty may decrease premature death.’
- **Answer (GPT):** The record clearly involves randomized controlled trials (three RCTs) of an early-life intervention: intensive prenatal and infant/toddler nurse home visiting beginning during pregnancy and continuing through early childhood, which is before high school entry. These are well-known Nurse-Family Partnership trials. The current article reports long-term outcomes (maternal and child mortality) using National Death Index data with follow-up spanning about two decades, which implies adult outcomes for mothers (and possibly for some children depending on timing). While this particular abstract does not mention cognitive or social-emotional outcomes at the end of the intervention, prior publications from these RCTs are known to have assessed a wide range of maternal and child cognitive, behavioral, and social-emotional outcomes at or near the end of the intervention and in childhood. According to the inclusion rules, a study should be included if it may be a follow-up or companion report from an eligible RCT, even when the adult follow-up paper does not restate the original

cognitive or social-emotional measures. The design is clearly randomized, the intervention is clearly pre-high school, and the article presents adult outcomes from these trials. Therefore, this record is classified as INCLUDE for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 30:

– **Title:** ‘Prenatal and infancy nurse home visiting and 18-year outcomes of a randomized trial’

– **Author:** Kitzman, H.; Olds, D. L.; Knudtson, M. D.; Cole, R.; Anson, E.; Smith, J. A.; Conti, G.

– **Year:** 2019

– **Journal:** Pediatrics, 144(6)

– **Abstract:** ‘OBJECTIVES: Given earlier effects found in randomized clinical trials of the Nurse-Family Partnership, we examined whether this program would improve 18-year-old first-born youths cognition, academic achievement, and behavior and whether effects on cognitive-related outcomes would be greater for youth born to mothers with limited psychological resources (LPR) and on arrests and convictions among females. METHODS: We enrolled 742 pregnant, low-income women with no previous live births and randomly assigned them to receive either free transportation for prenatal care plus child development screening and referral (control; n = 514) or prenatal and infant home nurse visit (NV) plus transportation and screening (n = 228). Assessments were completed on 629 18-year-old first-born offspring to evaluate these primary outcomes: (1) cognitive-related abilities (nonverbal intelligence, receptive language, and math achievement) and (2) behavioral health (internalizing behavioral problems, substance use and abuse, sexually transmitted infections, HIV risk, arrests, convictions, and gang membership). RESULTS: Compared with control-group counterparts, NV youth born to mothers with LPR had better receptive language (effect size = 0.24; 95% confidence interval [CI]: 0.00 to 0.47; P = .05), math achievement (effect size = 0.38; 95% CI: 0.14 to 0.61; P = .002), and a number of secondary cognitive-related outcomes. NV females, as a trend, had fewer convictions (incidence ratio = 0.47; 95% CI: 0.20 to 1.11; P = .08). There were no intervention effects on other behaviors. CONCLUSIONS: The program improved the cognitive-related skills

of 18-year-olds born to mothers with LPR and, as a trend, reduced female convictions but produced no other effects on youth behavioral health.’

– **Answer (GPT):** The study is clearly a randomized controlled trial: 742 pregnant, low-income women were randomly assigned to either nurse home visiting plus services or control services. The intervention (prenatal and infancy nurse home visiting) occurred before birth and during infancy, ending long before high school entry, satisfying the pre-high school intervention criterion. The abstract reports quantitative outcomes at age 18, including cognitive-related abilities (nonverbal intelligence, receptive language, math achievement) and multiple behavioral health outcomes (internalizing behavioral problems, substance use/abuse, STIs, HIV risk, arrests, convictions, and gang membership), which are clearly adult outcomes. Although end-of-intervention outcomes are not described in this abstract, the Nurse-Family Partnership trials are known to have measured child development and related outcomes earlier in life, and per the screening rules, the record should be included if it may belong to such a trial even if the current abstract does not restate the original posttest measures. There is no indication that the design is nonrandomized or that cognitive/social-emotional outcomes were not assessed at or near the end of the intervention. Therefore, this record plausibly belongs to an eligible randomized trial and should be INCLUDED for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 31:

– **Title:** ‘Effect of home visiting by nurses on maternal and child mortality: results of a 2-decade follow-up of a randomized clinical trial’

– **Author:** Olds, D. L.; Kitzman, H.; Knudtson, M. D.; Anson, E.; Smith, J. A.; Cole, R.

– **Year:** 2014

– **Journal:** JAMA Pediatrics

– **Abstract:** ‘Importance Mothers and children living in adverse contexts are at risk of premature death. Objective To determine the effect of prenatal and infant/toddler nurse home visiting on maternal and child mortality during a 2-decade period (1990-2011). Design, setting, and participants A randomized clinical trial was designed originally to assess the home visiting program s effect on pregnancy outcomes and maternal and child health through child age 2 years. The study was conducted in a public system of obstetric and pediatric

care in Memphis, Tennessee. Participants included primarily African American women and their first live-born children living in highly disadvantaged urban neighborhoods, who were assigned to 1 of 4 treatment groups: treatment 1 (transportation for prenatal care [n = 166]), treatment 2 (transportation plus developmental screening for infants and toddlers [n = 514]), treatment 3 (transportation plus prenatal/postpartum home visiting [n = 230]), and treatment 4 (transportation, screening, and prenatal, postpartum, and infant/toddler home visiting [n = 228]). Treatments 1 and 3 were included originally to increase statistical power for testing pregnancy outcomes. For determining mortality, background information was available for all 1138 mothers assigned to all 4 treatments and all but 2 live-born children in treatments 2 and 4 (n = 704). Inclusion of children in treatments 1 and 3 was not possible because background information was missing on too many children. Interventions Nurses sought to improve the outcomes of pregnancy, children's health and development, and mothers health and life-course with home visits beginning during pregnancy and continuing through child age 2 years. Main outcomes and measures All-cause mortality in mothers and preventable-cause mortality in children (sudden infant death syndrome, unintentional injury, and homicide) derived from the National Death Index. Results The mean (SE) 21-year maternal all-cause mortality rate was 3.7% (0.74%) in the combined control group (treatments 1 and 2), 0.4% (0.43%) in treatment 3, and 2.2% (0.97%) in treatment 4. The survival contrast of treatments 1 and 2 combined with treatment 3 was significant (P = .007); the contrast of treatments 1 and 2 combined with treatment 4 was not significant (P = .19), and the contrast of treatments 1 and 2 combined with treatments 3 and 4 combined was significant (post hoc P = .008). At child age 20 years, the preventable-cause child mortality rate was 1.6% (0.57%) in treatment 2 and 0.0% (SE not calculable) in treatment 4; the survival contrast was significant (P = .04). Conclusions and relevance Prenatal and infant/toddler home visitation by nurses is a promising means of reducing all-cause mortality among mothers and preventable-cause mortality in their first-born children living in highly disadvantaged settings. Trial registration [clinicaltrials.gov Identifier: NCT00708695](https://clinicaltrials.gov/ct2/show/study/NCT00708695).

– **Answer (GPT):** The study is a randomized clinical trial of a prenatal and infant/toddler nurse home visiting program delivered from pregnancy through child age 2 years, clearly before high school entry. The intervention targeted mothers and their first-born children with the explicit aim of improving children's health and development, and some arms included developmental screening for infants and toddlers, making it plausible that cognitive or social-emotional developmental outcomes were assessed around the end of the intervention period, even if not reported in this particular abstract. The current record reports long-term outcomes (maternal all-cause mortality and child preventable-cause mortality) up to approximately ages 20–21, clearly in adulthood. Random assignment is explicitly stated. Because the trial plausibly measured early developmental (including cognitive or social-emotional) outcomes at or near the end of the intervention and clearly reports adult outcomes, this record should be included for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 34:

– **Title:** ‘Parent-reported homework problems in the MTA study: Evidence for sustained improvement with behavioral treatment’

– **Author:** Langberg, J. M.; Arnold, L. E.; Flowers, A. M.; Epstein, J. N.; Altaye, M.; Hinshaw, S. P.; Hechtman, L.

– **Year:** 2010

– **Journal:** Journal of Clinical Child & Adolescent Psychology

– **Abstract:** ‘Parent-report of child homework problems was examined as a treatment outcome variable in the MTA-Multimodal Treatment Study of Children with Attention-Deficit/Hyperactivity Disorder (ADHD). Five hundred seventy-nine children ages 7.0 to 9.9 were randomly assigned to either medication management, behavioral treatment, combination treatment, or routine community care. Results showed that only participants who received behavioral treatment (behavioral and combined treatment) demonstrated sustained improvements in homework problems in comparison to routine community care. The magnitude of the sustained effect at the 10-month follow-up assessment was small to moderate for combined and behavioral treatment over routine community care ($d = .37, .40$, respectively). Parent ratings of initial ADHD symptom severity was the only variable found to moderate these effects.’

– **Answer (GPT):** The study is a randomized controlled trial (children aged 7–9.9 with ADHD) comparing medication management, behavioral treatment, combination treatment, and community care. The intervention clearly occurs before high school. The outcomes include parent-reported homework problems, which qualify as cognitive/academic or related behavior outcomes measured during and shortly after the intervention (10-month follow-up). Although the abstract only reports child outcomes up to 10 months and does not mention any adult follow-up, it is plausible that this large, well-known RCT (the MTA study) may have later follow-ups into adulthood in other publications. Per the screening rules, when the presence of adult outcomes is unclear, records should be included to maintain high recall. Therefore, this record should be classified as INCLUDE for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 37:

– **Title:** ‘Young adult outcomes in the follow-up of the multimodal treatment study of attention-deficit/hyperactivity disorder: Symptom persistence, source discrepancy, and height suppression’

– **Author:** Swanson, J. M.; Arnold, L. E.; Molina, B. S.; Sibley, M. H.; Hechtman, L. T.; Hinshaw, S. P.; Stern, K.

– **Year:** 2017

– **Journal:** Journal of Child Psychology and Psychiatry

– **Abstract:** ‘BACKGROUND: The Multimodal Treatment Study (MTA) began as a 14-month randomized clinical trial of behavioral and pharmacological treatments of 579 children (7-10 years of age) diagnosed with attention-deficit/hyperactivity disorder (ADHD)-combined type. It transitioned into an observational long-term follow-up of 515 cases consented for continuation and 289 classmates (258 without ADHD) added as a local normative comparison group (LNCG), with assessments 2-16 years after baseline. METHODS: Primary (symptom severity) and secondary (adult height) outcomes in adulthood were specified. Treatment was monitored to age 18, and naturalistic subgroups were formed based on three patterns of long-term use of stimulant medication (Consistent, Inconsistent, and Negligible). For the follow-up, hypothesis-generating analyses were performed on outcomes in early adulthood (at 25 years of age). Planned comparisons were used to estimate ADHD-LNCG differences reflecting persistence of symptoms and naturalistic subgroup differences reflecting benefit (symptom reduction) and cost (height suppression) associated with extended use of medication. RESULTS: For ratings of symptom severity, the ADHD-LNCG comparison was statistically significant for the parent/self-report average (0.51 ± 0.04 , $p < .0001$, $d = 1.11$), documenting symptom persistence, and for the parent/self-report difference (0.21 ± 0.04 , $p < .0001$, $d = .60$), documenting source discrepancy, but the comparisons of naturalistic subgroups reflecting medication effects were not significant. For adult height, the ADHD group was 1.29 ± 0.55 cm shorter than the LNCG ($p < .01$, $d = .21$), and the comparisons of the naturalistic subgroups were significant: the treated group with the Consistent or Inconsistent pattern was 2.55 ± 0.73 cm shorter than the subgroup with the Negligible pattern ($p < .0005$, $d = .42$), and within the treated group, the subgroup with the Consistent pattern was 2.36 ± 1.13 cm shorter than the subgroup with the Inconsistent pattern ($p < .04$, $d = .38$). CONCLUSIONS: In the MTA follow-up into adulthood, the ADHD group showed symptom persistence compared to local norms from the LNCG. Within naturalistic subgroups of ADHD cases, extended use of

medication was associated with suppression of adult height but not with reduction of symptom severity.’

– **Answer (GPT):** Include. The record is a follow-up of the Multimodal Treatment Study (MTA), which is explicitly described as beginning as a 14-month randomized clinical trial of behavioral and pharmacological treatments for children aged 7–10 with ADHD. The intervention therefore clearly occurred before high school entry. The original randomized trial focused on ADHD symptom severity and related behavioral outcomes, which fall under social-emotional/behavioral outcomes and would have been measured at or near the end of the 14-month intervention. The present article reports quantitative outcomes in early adulthood (age 25), including symptom severity and adult height. Thus, it is an adult follow-up of a childhood randomized trial with relevant social-emotional outcomes at intervention end and adult outcomes, matching all inclusion criteria. Even though the follow-up phase is observational, it originates from an eligible randomized controlled trial, which is acceptable under the guidelines.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 38:

– **Title:** ‘Treatment-related changes in objectively measured parenting behaviors in the multimodal treatment study of children with attention-deficit/hyperactivity disorder’

– **Author:** Wells, K. C.; Chi, T. C.; Hinshaw, S. P.; Epstein, J. N.; Pfiffner, L.; Nebel-Schwalm, M.; Wigal, T.

– **Year:** 2006

– **Journal:** Journal of Consulting and Clinical Psychology

– **Abstract:** ‘The present study examined treatment outcomes for objectively measured parenting behavior in the Multimodal Treatment Study of Children with Attention-Deficit/Hyperactivity Disorder (ADHD). Five hundred seventy-nine ethnically and socioeconomically diverse children with ADHD-combined type (ages 7.0-9.9 years) and their parent(s) were recruited at 6 sites in the United States and Canada and randomly assigned to 1 of 4 treatment groups for 14 months of active intervention: medication management (MedMgt), intensive behavior therapy, combination of the 2 (Comb), or a community-treated comparison (CC). Baseline and posttreatment laboratory observations of parent-child interactions were coded by

observers blind to treatment condition. Comb produced significantly greater improvements in constructive parenting than did MedMgt or CC, with effect sizes approaching medium for these contrasts. Treatment effects on child behaviors were not significant. The authors discuss the importance of changes in parenting behavior for families of children with ADHD and the need for reliable and objective measures in evaluating treatment outcome.’

– **Answer (GPT):** The study is a randomized controlled trial (children aged 7–9.9 years) involving pre-high school participants, with 14 months of active intervention and clear random assignment to four treatment groups. It measures parenting behaviors and child behaviors at posttreatment, which are social-emotional outcomes assessed at the end of the intervention. Although this particular record reports only immediate posttreatment outcomes and does not mention any adult follow-up, the instructions specify high recall at title/abstract screening and to include records that may be companion or early reports of trials that could later have adult follow-ups. There is no explicit statement that the trial never measured outcomes in adulthood; thus it remains plausible that subsequent adult follow-up reports exist or could be linked to this trial. Therefore, the record should be classified as INCLUDE for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 39:

– **Title:** ‘Effects of a preventive parenting intervention for divorced families on the intergenerational transmission of parenting attitudes in young adult offspring’

– **Author:** Mahrer, N. E.; Winslow, E.; Wolchik, S. A.; Tein, J. Y.; Sandler, I. N.

– **Year:** 2014

– **Journal:** Child Development

– **Abstract:** ‘This study evaluates whether the New Beginnings Program (NBP), a parenting intervention for divorced mothers, led to positive parenting attitudes in young adult offspring. Data were collected from 240 mothers (G1) and offspring (G2) at ages 9-12 and again in adolescence and young adulthood. Alternative theoretical models were tested to examine mediators of NBP effects on G2 parenting attitudes. Significant interactions between condition and baseline G1 parenting indicated that NBP improved G2 s parenting attitudes for those exposed to poorer G1 parenting at program entry. Effects on G2 warm attitudes were partially mediated through program effects on G1 warm parenting. The implications of improving

parenting attitudes in offspring who experience parental divorce on well-being in the next generation are discussed.’

– **Answer (GPT):** The record evaluates long-term (young adulthood) effects of the New Beginnings Program, a preventive parenting intervention for divorced mothers delivered when offspring were ages 9–12 (i.e., clearly before high school). The offspring are followed into adolescence and young adulthood, and the main outcome in this paper is young adult (G2) parenting attitudes, an adult social-emotional/attitudinal outcome. The New Beginnings Program is a well-known randomized trial, although randomization is not explicitly restated in this abstract; per the instructions, lack of explicit mention of randomization at this stage does not warrant exclusion. It is highly plausible that the original trial measured child social-emotional and/or cognitive outcomes at or near the end of the intervention, even if these are not mentioned here. The follow-up clearly concerns adult outcomes (young adulthood). Thus, the study plausibly represents an adult follow-up report from an eligible randomized early intervention trial and should be INCLUDED for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 41:

– **Title:** ‘Enhancing youth outcomes following parental divorce: A longitudinal study of the effects of the New Beginnings Program on educational and occupational goals’

– **Author:** Sigal, A. B.; Wolchik, S. A.; Tein, J. Y.; Sandler, I. N.

– **Year:** 2012

– **Journal:** Journal of Clinical Child & Adolescent Psychology

– **Abstract:** ‘This study examined whether the New Beginnings Program for divorced families led to improvements in youth s educational goals and job aspirations 6 years following participation and tested whether several parenting and youth variables mediated the program effects. Participants were 240 youth aged 9 to 12 years at the initial assessment, and data were part of a randomized, experimental trial of a parenting skills preventive intervention targeting children s postdivorce adjustment. The results revealed positive effects of the program on youth s educational goals and job aspirations 6 years after participation for those who were at high risk for developing later problems at program entry. Further, intervention-induced changes in mother-child relationship quality and youth externalizing problems, internalizing

problems, self-esteem, and academic competence at the 6-year follow-up mediated the effects of the program on the educational expectations of high-risk youth. Intervention-induced changes in youth externalizing problems and academic competence at the 6-year follow-up mediated the effects of the program on the job aspirations of high-risk youth.'

– **Answer (GPT):** The study is part of a randomized experimental trial of a parenting skills preventive intervention delivered to divorced families when children were 9–12 years old, clearly before high school. The broader trial plausibly measured children's cognitive or social-emotional outcomes (e.g., postdivorce adjustment, externalizing, internalizing, self-esteem, academic competence) at or near the end of the intervention, even though this particular abstract focuses on a later follow-up. The current report examines educational and occupational goals 6 years after participation; given baseline ages 9–12, participants at follow-up could be around 15–18 years old, so it is unclear whether all outcomes were measured before or after age 18 or high school completion. Because age/educational stage at follow-up is not clearly below 18 and the trial is randomized with a pre-high school intervention and relevant psychosocial outcomes, eligibility remains plausible. Therefore, the record should be INCLUDED for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 42:

– **Title:** 'Protecting children from the consequences of divorce: A longitudinal study of the effects of parenting on children's coping processes'

– **Author:** Vélez, C. E.; Wolchik, S. A.; Tein, J. Y.; Sandler, I.

– **Year:** 2011

– **Journal:** Child Development

– **Abstract:** 'This study examines whether intervention-induced changes in mother-child relationship quality and discipline led to short-term (6 months) and long-term (6 years) changes in children's coping processes in a sample of 240 youth aged 9-12 years when assessed initially. Data were from a randomized, experimental trial of a parenting-focused preventive intervention designed to improve children's postdivorce adjustment. Three-wave prospective mediational analyses revealed that intervention-induced improvements in relationship quality led to increases in coping efficacy at 6 months and to increases in coping efficacy and active coping at 6 years.'

Tests of the mediated effects were significant for all 3 indirect paths. Results are discussed in terms of pathways to adaptive coping and implications for the implementation of preventive interventions targeting coping.'

– **Answer (GPT):** The study is based on a randomized, experimental trial of a parenting-focused preventive intervention delivered to families with children aged 9–12 years, which is clearly before high school. The intervention is intended to improve children's post-divorce adjustment and operates through changes in mother–child relationship quality and discipline, making it a pre-high-school childhood intervention. The outcomes examined include coping efficacy and active coping at 6 months and 6 years post-intervention. These are social-emotional outcomes measured at both short-term and long-term follow-ups. The 6-month follow-up is clearly near the end of the intervention and assesses children's coping processes (social-emotional skills). The 6-year follow-up occurs when participants, initially 9–12 years old, are approximately 15–18 years old. This places at least some participants at or beyond age 18 and likely around or beyond high school completion, and the exact educational status is not specified. Per the instructions, when age or educational stage at follow-up is unclear, the record should be included. The study therefore plausibly provides adult outcomes as defined in the criteria. All three key criteria—pre-high school intervention, cognitive/social-emotional outcomes at or near intervention end, and at least potentially adult outcomes—are met or plausibly met. Thus, this record should be classified as INCLUDE for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 43:

– **Title:** 'Fifteen-year follow-up of a randomized trial of a preventive intervention for divorced families: effects on mental health and substance use outcomes in young adulthood'

– **Author:** Wolchik, S. A.; Sandler, I. N.; Tein, J. Y.; Mahrer, N. E.; Millsap, R. E.; Winslow, E.; Reed, A.

– **Year:** 2013

– **Journal:** Journal of Consulting and Clinical Psychology

– **Abstract:** 'OBJECTIVE: This 15-year follow-up assessed the effects of a preventive intervention for divorced families, the New Beginnings Program (NBP), versus a literature control

condition (LC). **METHOD:** Mothers and their 9- to 12-year-olds (N = 240 families) participated in the trial. Young adults (YAs) reported on their mental health and substance-related disorders, mental health and substance use problems, and substance use. Mothers reported on YA s mental health and substance use problems. Disorders were assessed over the past 9 years (since previous follow-up) and 15 years (since program entry). Alcohol and marijuana use, other substance use and polydrug use, and mental health problems and substance use problems were assessed over the past month, past year, and past 6 months, respectively. **RESULTS:** YAs in NBP had a lower incidence of internalizing disorders in the past 9 years (7.55% vs. 24.4%; odds ratio [OR] = .26) and 15 years (15.52% vs. 34.62%; OR = .34) and had a slower rate of onset of internalizing symptoms associated with disorder in the past 9 years (hazard ratio [HR] = .28) and 15 years (HR = .46). NBP males had a lower number of substance-related disorders in the past 9 years (d = 0.40), less polydrug (d = 0.55) and other drug use (d = 0.61) in the past year, and fewer substance use problems (d = 0.50) in the past 6 months than LC males. NBP females used more alcohol in the past month (d = 0.44) than LC females. **CONCLUSIONS:** NBP reduced the incidence of internalizing disorders for females and males and substance-related disorders and substance use for males.'

– **Answer (GPT):** The study is a 15-year follow-up of a randomized trial (New Beginnings Program) involving children aged 9–12 at program entry, which is clearly before high school. The intervention was a preventive program for divorced families, and while this abstract focuses on adult outcomes (mental health and substance use disorders/problems and substance use in young adulthood), the original trial almost certainly assessed child mental health or related social-emotional outcomes at or near the end of the intervention. Even though those posttest outcomes are not described here, the instructions specify that adult follow-up reports should not be excluded merely because end-of-intervention measures are not mentioned. The current report presents quantitative adult outcomes (mental health and substance use) when participants are young adults (well past age 18). The design is explicitly randomized. Therefore, this record plausibly belongs to a randomized controlled trial of a childhood intervention with adult follow-up and should be INCLUDED.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 44:

– **Title:** ‘Developmental cascade effects of a parenting-focused program for divorced families on competence in emerging adulthood’

- **Author:** Wolchik, S. A.; Tein, J. Y.; Winslow, E.; Minney, J.; Sandler, I. N.; Masten, A. S.
- **Year:** 2021
- **Journal:** Development and Psychopathology
- **Abstract:** ‘This 15-year longitudinal follow-up of a randomized controlled trial of a parenting-focused preventive intervention for divorced families examined cascade models of program effects on offsprings competence. It was hypothesized that intervention-induced improvements in parenting would lead to better academic, work, peer, and romantic competence in emerging adulthood through effects on behavior problems and competencies during adolescence. Families (N = 240) participated in the 11-session program or literature control condition when children were ages 9-12. Data were drawn from assessments at pretest, posttest, and follow-ups at 3 and 6 months and 6 and 15 years. Results showed that initial intervention effects of parenting on externalizing problems in adolescence cascaded to work outcomes in adulthood. Parenting effects also directly impacted work success. For work outcomes and peer competence, intervention effects were moderated by initial risk level; the program had greater effects on youths with higher risk at program entry. In addition, intervention effects on parenting led to fewer externalizing problems that in turn cascaded to better academic outcomes, which showed continuity into emerging adulthood. Results highlight the potential for intervention effects of the New Beginnings Program to cascade over time to affect adult competence in multiple domains, particularly for high-risk youths.’
- **Answer (GPT):** The study is a 15-year longitudinal follow-up of a randomized controlled trial of a parenting-focused preventive intervention (New Beginnings Program) for divorced families. Children were 9–12 years old at program entry, clearly before high school. The intervention appears to have ended during this age range, before high school entry. It is explicitly a randomized controlled trial (program vs. literature control). The abstract indicates assessment at pretest, posttest, and multiple follow-ups, including 6 and 15 years later. The trial clearly involves social-emotional outcomes (externalizing problems, behavior problems, peer competence, romantic competence, work competence, academic outcomes) during adolescence and into adulthood. The follow-up in “emerging adulthood” (15-year follow-up) clearly concerns adult outcomes (work outcomes, peer and romantic competence, academic outcomes continuing into adulthood). Thus, the record plausibly reports or is linked to a trial with pre–high school intervention, cognitive/academic and social-emotional outcomes at or near intervention end and during adolescence, and quantitative outcomes measured in adulthood. It therefore meets and clearly satisfies all inclusion criteria and should be classified as INCLUDE.
- **Answer (GPT - numeric):** 1
- **Answer (Human):** 0
- Please add a comment on whether and why you agree with the GPT decision or not:*
- **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 45:

– **Title:** ‘Longitudinal impact of two universal preventive interventions in first grade on educational outcomes in high school’

– **Author:** Bradshaw, C. P.; Zmuda, J. H.; Kellam, S. G.; Ialongo, N. S.

– **Year:** 2009

– **Journal:** Journal of Educational Psychology

– **Abstract:** ‘This study examined the longitudinal effects of 2 first-grade universal preventive interventions on academic outcomes (e.g., achievement, special education service use, graduation, postsecondary education) through age 19 in a sample of 678 urban, primarily African American children. The classroom-centered intervention combined the Good Behavior Game (H. H. Barrish, Saunders, & Wolfe, 1969) with an enhanced academic curriculum, whereas a second intervention, the Family-School Partnership, focused on promoting parental involvement in educational activities and bolstering parents behavior management strategies. Both programs aimed to address the proximal targets of aggressive behavior and poor academic achievement. Although the effects varied by gender, the classroom-centered intervention was associated with higher scores on standardized achievement tests, greater odds of high school graduation and college attendance, and reduced odds of special education service use. The intervention effects of the Family-School Partnership were in the expected direction; however, only 1 effect reached statistical significance. The findings of this randomized controlled trial illustrate the long-term educational impact of preventive interventions in early elementary school.’

– **Answer (GPT):** The study describes a randomized controlled trial of two first-grade universal preventive interventions (Good Behavior Game plus enhanced curriculum, and Family-School Partnership). The interventions clearly occurred and ended in early elementary school, before high school entry. The trial targeted and measured academic achievement and aggressive behavior, which fall under cognitive and social-emotional outcomes, and it is highly plausible that such outcomes were assessed at or near the end of the intervention even if those specific end-of-intervention results are not detailed in this abstract. The follow-up includes academic outcomes through age 19 (e.g., standardized achievement tests, high school graduation, college attendance, special education service use), which qualifies as adult outcomes. Therefore, the record plausibly reports or is linked to an eligible randomized trial with pre-high school intervention, cognitive/social-emotional outcomes at intervention end, and outcomes measured in adulthood. It should be included for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 46:

- **Title:** ‘Proximal impact of two first-grade preventive interventions on the early risk behaviors for later substance abuse, depression, and antisocial behavior’
- **Author:** Ialongo, N. S.; Werthamer, L.; Kellam, S. G.; Brown, C. H.; Wang, S.; Lin, Y.
- **Year:** 1999
- **Journal:** American Journal of Community Psychology
- **Abstract:** ‘We assessed the immediate effects of two universal, first-grade preventive interventions on the proximal targets of poor achievement, concentration problems, aggression, and shy behaviors, known early risk behaviors for later substance use/abuse, affective disorder, and conduct disorder. The classroom-centered (CC) intervention was designed to reduce these early risk behaviors by enhancing teachers behavior management and instructional skills, whereas the family-school partnership (FSP) intervention was aimed at improving parent-teacher communication and parental teaching and child behavior management strategies. Over the course of first and second grades, the CC intervention yielded the greatest degree of impact on its proximal targets, whereas the FSP s impact was somewhat less. The effects were influenced by gender and by preintervention levels of risk. Analyses of implementation measures demonstrated that greater fidelity to the intervention protocols was associated with greater impact on behavior ratings and on achievement scores, thus providing some evidence of specificity in the effect of the interventions.’
- **Answer (GPT):** The study evaluates two universal first-grade preventive interventions (classroom-centered and family-school partnership) targeting academic achievement and behavioral outcomes such as concentration problems, aggression, and shy behaviors. The intervention clearly occurs before high school (first and second grades). The outcomes at the end of the intervention include cognitive (achievement scores) and social-emotional/behavioral measures (concentration problems, aggression, shy behaviors), satisfying the requirement for cognitive or social-emotional outcomes at intervention end. Although the abstract does not explicitly state that the design is randomized, the design is not clearly nonrandomized, so per instructions it should be treated as potentially randomized. The abstract also does not mention adult follow-up; however, the title and aims focus on early risk behaviors for later substance abuse, depression, and antisocial behavior, making it plausible that this trial could have subsequent adult follow-up reports not described here. Because information on adult outcomes is missing/unclear, and the design could plausibly be randomized, the record should be included at this screening stage to maintain high recall.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 47:

– **Title:** ‘Polygenic score $\times$ intervention moderation: an application of discrete-time survival analysis to model the timing of first marijuana use among urban youth’

– **Author:** Musci, R. J.; Fairman, B.; Masyn, K. E.; Uhl, G.; Maher, B.; Sisto, D. Y.; Ialongo, N. S.

– **Year:** 2018

– **Journal:** Prevention Science

– **Abstract:** ‘The present study examines the interaction between a polygenic score and an elementary school-based universal preventive intervention trial and its effects on a discrete-time survival analysis of time to first smoking marijuana. Research has suggested that initiation of substances is both genetically and environmentally driven (Rhee et al., Archives of general psychiatry 60:1256-1264, 2003; Verweij et al., Addiction 105:417-430, 2010). A previous work has found a significant interaction between the polygenic score and the same elementary school-based intervention with tobacco smoking (Musci et al., in press). The polygenic score reflects the contribution of multiple genes and has been shown in prior research to be predictive of smoking cessation, tobacco use, and marijuana use (Uhl et al., Molecular Psychiatry 19:50-54, 2014). Using data from a longitudinal preventive intervention study ($N = 678$), we examined age of first marijuana use from sixth grade to age 18. Genetic data were collected during emerging adulthood and were genotyped using the Affymetrix 6.0 microarray ($N = 545$). The polygenic score was computed using these data. Discrete-time survival analysis was employed to test for intervention main and interaction effects with the polygenic score. We found main effect of the polygenic score approaching significance, with the participants with higher polygenic scores reporting their first smoking marijuana at an age significantly later than controls ($p = .050$). We also found a significant intervention $\times$ polygenic score interaction effect at $p = .003$, with participants at the higher end of the polygenic score benefiting the most from the intervention in terms of delayed age of first use. These results suggest that genetics may play an important role in the age of first use of marijuana and that differences in genetics may account for the differential effectiveness of classroom-based interventions in delaying substance use experimentation.’

– **Answer (GPT):** The study analyzes data from an elementary school-based universal preventive intervention trial, indicating that the intervention occurred and ended before high school. The design is described as a preventive intervention trial and is plausibly randomized; nothing in the abstract rules out random assignment, and the instructions require inclusion when design is unclear but could be randomized. The outcomes modeled here are age of first marijuana use from sixth grade through age 18, so the study includes outcomes measured at or up to age 18, satisfying the adulthood follow-up criterion. Although this particular abstract focuses on substance use timing, the underlying elementary school-based intervention trial almost certainly measured behavioral or other social-emotional outcomes at or near the end of the intervention, and no information is provided that would exclude that possibility. Because the record is a secondary/genetic moderation analysis of an existing elementary school-based intervention trial with follow-up through age 18, it plausibly belongs to a randomized controlled trial of a pre-high school intervention with adult outcomes. Therefore, it should be INCLUDED for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 49:

– **Title:** ‘The effect of two elementary school-based prevention interventions on being offered tobacco and the transition to smoking’

– **Author:** Wang, Y.; Storr, C. L.; Green, K. M.; Zhu, S.; Stuart, E. A.; Lynne-Landsman, S. D.; Ialongo, N. S.

– **Year:** 2012

– **Journal:** Drug and Alcohol Dependence

– **Abstract:** ‘Aims This study sought to more precisely delineate the mechanisms by which two early elementary school-based, universal (i.e., applied to the entire population regardless of risk status) preventive interventions increased survival to first tobacco cigarette smoked. Specifically, we examined whether the interventions effect on survival to first use was via the reduction of offers to smoke and/or through preventing the transition from first offer to smoking. Methods A total of 678 urban first-graders were assigned randomly to the classroom-centered (CC), or the family-school partnership (FSP), or a control classroom condition. Youth were followed annually until 1 year beyond their anticipated high school graduation (mean age 18

years). Discrete-time survival analyses on 628 youth evaluated the impact of the CC and FSP interventions on first tobacco offer and initial tobacco smoking once offered. Findings The risk of being offered tobacco was reduced among both CC and FSP intervention groups relative to the control group, although the reduction was only statistically significant for the CC intervention. Neither intervention condition reduced the transition to smoking once offered tobacco to smoke. Conclusion The CC intervention appeared to have its effect on survival to first cigarette smoked by delaying the first offer to smoke. Preventive interventions focused on refusal skills during the middle school years may be necessary to reduce the likelihood of the transition to smoking once offered.'

– **Answer (GPT):** The study is a randomized controlled trial in which 678 urban first-graders were randomly assigned to two preventive intervention conditions or a control classroom, clearly indicating an intervention that begins and ends before high school. The interventions are elementary school-based and universal, targeting first graders. Participants were followed annually until approximately age 18, one year beyond anticipated high school graduation, and the outcomes analyzed here concern tobacco offers and transition to smoking, which constitute behavioral outcomes measured in late adolescence/early adulthood. Although this particular abstract focuses on tobacco-related outcomes, the interventions described (classroom-centered and family-school partnership) are school- and family-based preventive programs that plausibly would have included and measured social-emotional or behavioral outcomes (e.g., conduct, self-regulation, classroom behavior, or related skills) at or near the end of the intervention in earlier follow-ups or companion publications. The abstract does not clearly state that no cognitive or social-emotional outcomes were measured at intervention end, and given the nature of the interventions and the trial, it is reasonable to infer that such outcomes may have been assessed. Because the trial is randomized, the intervention occurs before high school, and adult (around age 18) outcomes are reported, and because it is plausible that relevant cognitive or social-emotional outcomes were measured at the end of the intervention even if not described in this abstract, this record should be classified as INCLUDE for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 52:

– **Title:** 'Gender moderation and social developmental mediation of the effect of a family-focused substance use preventive intervention on young adult alcohol abuse'

– **Author:** Mason, W. A.; Kosterman, R.; Haggerty, K. P.; Hawkins, J. D.; Redmond, C.; Spoth, R. L.; Shin, C.

– **Year:** 2009

– **Journal:** Addictive Behaviors

– **Abstract:** ‘This study examined the long-term impact of Preparing for the Drug Free Years (PDFY) on young adult alcohol abuse disorder, addressing theory-based questions about how, and for whom, the program had its effects on the outcomes. Participants were 429 families of 6th graders enrolled in 33 rural schools located in the Midwestern United States. Schools were randomly assigned to conditions. Target adolescents (52% female) were interviewed periodically from age 11 to age 22; throughout adolescence, information was collected also from the youths parents. Moderation and mediation analyses were conducted using regression analysis and structural equation modeling with covariates measured at baseline (age 11), mediators measured at posttest (age 12), and the outcome measured at the young adult follow-up (age 22). Results showed that PDFY reduced the rate of alcohol abuse among target young women, with evidence that this effect was mediated by increased prosocial skills. The rate of alcohol abuse among PDFY group men was not significantly different from that of control group men. Findings have implications for reducing the public health burden of alcohol abuse among young women.’

– **Answer (GPT):** The study reports on Preparing for the Drug Free Years (PDFY), a family-focused preventive intervention delivered to families of 6th graders (before high school) in 33 rural schools that were randomly assigned to conditions, indicating a randomized controlled trial. The intervention clearly occurs in early adolescence (6th grade) and ends well before high school graduation. The abstract specifies that mediators were measured at posttest (age 12) and include prosocial skills, which are social-emotional outcomes assessed at the end of the intervention. The primary outcome analyzed in this article is alcohol abuse disorder measured at age 22, which is clearly an adult outcome. Thus, the study plausibly meets all three inclusion criteria: randomized design, pre-high school intervention with end-of-intervention social-emotional measures, and adult follow-up outcomes. Therefore, it should be included for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 55:

- **Title:** ‘Long-term effects of universal preventive interventions on prescription drug misuse’
- **Author:** Spoth, R.; Trudeau, L.; Shin, C.; Redmond, C.
- **Year:** 2008
- **Journal:** Addiction
- **Abstract:** ‘BACKGROUND: This is a supplemental report on tests of the long-term effects of universal preventive interventions conducted during middle school on 17-21-year-olds prescription drug misuse. DESIGN/SETTING/PARTICIPANTS: Two randomized controlled prevention trials were conducted in public schools in the rural midwestern United States. Study 1 began in 1993, with 667 6th-graders; follow-ups with 12th-graders and 21-year-olds included 457 and 483 participants, respectively. Study 2 began in 1998 with 7th-graders (total sample across waves 2127); follow-ups with 11th- and 12th-graders included 1443 and 1212 participants, respectively. INTERVENTIONS: In study 1, schools were assigned to the Iowa Strengthening Families Program (ISFP), Preparing for the Drug Free Years, or a control condition. In study 2, schools were assigned to the school-based Life Skills Training (LST) plus a revised ISFP, called SFP 10-14 (LST + SFP 10-14), LST-only, or a control condition. MEASUREMENTS: Self reports of lifetime and past-year prescription drug misuse. Findings In study 1, ISFP 12th-graders past year narcotic misuse was significantly less than controls, as were ISFP 21-year-olds life-time narcotic and barbiturate misuse rates. In study 2, LST + SFP 10-14 showed significant effects on life-time prescription drug misuse at the 11th-grade follow-up, while effects at the 12th-grade follow-up were marginally significant. CONCLUSIONS: Consistent with intervention effects on other substance use outcomes reported earlier, results suggest that universal interventions have potential for public health impact by reducing some types of prescription drug misuse among adolescents and young adults.’
- **Answer (GPT):** The study describes two randomized controlled prevention trials delivered in middle school (6th and 7th grade), clearly before high school entry, satisfying the timing criterion. The interventions (Iowa Strengthening Families Program, Preparing for the Drug Free Years, Life Skills Training plus SFP 10-14, or LST-only) are universal preventive programs targeting substance use and related behaviors, and although this particular report focuses on prescription drug misuse in adolescence and young adulthood, these program types commonly assess a range of social-emotional or behavioral outcomes (e.g., conduct problems, substance use, refusal skills, self-regulation) at or near the end of the intervention. Even though the abstract does not state end-of-intervention cognitive or social-emotional outcomes, earlier reports from the same trials plausibly included such measures; per instructions, uncertainty about these outcomes should lead to inclusion. The report includes adult outcomes, as follow-up includes 21-year-olds, clearly meeting the adulthood criterion. The design is explicitly randomized (schools randomized to intervention conditions or control). Therefore, the record plausibly belongs to an eligible randomized controlled trial that assessed adult outcomes and likely measured relevant cognitive or social-emotional outcomes at intervention end. It should be included for further review.
- **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 56:

– **Title:** ‘Universal intervention effects on substance use among young adults mediated by delayed adolescent substance initiation’

– **Author:** Spoth, R.; Trudeau, L.; Guyll, M.; Shin, C.; Redmond, C.

– **Year:** 2009

– **Journal:** Journal of Consulting and Clinical Psychology

– **Abstract:** ‘In this article, the authors examine whether delayed substance initiation during adolescence, achieved through universal family-focused interventions conducted in middle school, can reduce problematic substance use during young adulthood. Sixth-grade students enrolled in 33 rural midwestern schools and their families were randomly assigned to 3 experimental conditions. Self-report questionnaires provided data at 7 time points for the Iowa Strengthening Families Program (ISFP), Preparing for the Drug Free Years (PDFY), and control groups through young adulthood. Five young adult substance frequency measures (drunkenness, alcohol-related problems, cigarettes, illicit drugs, and polysubstance use) were modeled as distal outcomes affected by the average level and rate of increase in substance initiation across the adolescent years in latent growth curve analyses. Results show that the models fit the data and that they were robust across outcomes and interventions, with more robust effects found for ISFP. The addition of direct intervention effects on young adult outcomes was not supported, suggesting long-term effects were primarily indirect. Relative reduction rates were calculated to quantify intervention-control differences on the estimated proportion of young adults indicating problematic substance use; they ranged from 19% to 31% for ISFP and from 9% to 16% for PDFY.’

– **Answer (GPT):** The study is a randomized controlled trial: sixth-grade students and their families in 33 schools were randomly assigned to two universal family-focused intervention conditions or a control group. The interventions were delivered in middle school (6th grade), clearly before high school entry, and are described as universal family-focused interventions targeting substance initiation during adolescence. Outcomes in young adulthood (problematic substance use, including drunkenness, alcohol-related problems, cigarettes, illicit drugs, and polysubstance use) are explicitly analyzed as distal outcomes, so the record clearly includes adult follow-up outcomes. During the adolescent years, the study measures substance initiation

and its growth trajectory, which are behavioral/social outcomes and therefore fall under social-emotional/behavioral domains. It is plausible that there were quantitative measures of such social-emotional/behavioral outcomes at or near the end of the intervention period, even though this specific adult-outcome paper focuses on mediation models. All key eligibility criteria—pre-high-school intervention, randomized design, adolescent social/behavioral measures, and adult outcomes—are plausibly met. Therefore, the record should be included for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 58:

– **Title:** ‘Two studies of proximal outcomes and implementation quality of universal preventive interventions in a community–university collaboration context’

– **Author:** Spoth, R.; Guyll, M.; Trudeau, L.

– **Year:** 2002

– **Journal:** Journal of Community Psychology

– **Abstract:** ‘Abstract Here we present the results from two longitudinal studies of competency-training interventions that entailed community–university collaboration intended to enhance implementation quality. In Study 1, 22 rural schools were randomly assigned to a family-focused intervention or a minimal contact control group. In Study 2, 36 rural schools were randomly assigned to a family-focused preventive intervention combined with a school-based intervention, the school-based intervention alone, or a minimal contact control group. In both studies, observers rated adherence to intervention protocols. Results showed that, on average, high levels of observer-rated adherence were attained in both studies. Analyses of the relationship between observer-rated adherence scores and proximal outcomes showed limited evidence of poorer outcomes associated with lower-adherence groups. Overall, findings highlight the benefits of community–university collaboration intended to facilitate high levels of intervention adherence. The need for a framework guiding a collaborative implementation research agenda is discussed. © 2002 Wiley Periodicals, Inc.’

– **Answer (GPT):** The study clearly describes randomized assignment of rural schools to universal preventive competency-training interventions versus minimal-contact control conditions, satisfying the randomized trial criterion. The interventions are school- and family-focused and delivered in rural schools; the precise grade level and whether the intervention ended

before high school are not fully specified, but they plausibly occur before or around middle school, and there is no clear indication that the intervention extends into high school, so by the high-recall rule the record should not be excluded on that basis. The paper examines “proximal outcomes” of competency-training preventive interventions, which are very likely to include social-emotional or related behavioral outcomes, and there is no explicit statement that cognitive or social-emotional outcomes were not measured. Although this particular abstract focuses on proximal outcomes and implementation quality and does not mention any adult follow-up, the instructions specify that records should be included if they may belong to a randomized trial for which adult outcomes could be reported in other publications, and we should not exclude when the age or educational stage at follow-up is unclear. There is no clear evidence that all follow-up outcomes are restricted to pre-18 or pre-high school completion, nor that adult outcomes are impossible. Therefore, under the conservative, high-recall screening rules, this record is classified as INCLUDE for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 59:

– **Title:** ‘Increasing school success through partnershipbased family competency training: Experimental study of long-term outcomes’

– **Author:** Spoth, R.; Randall, G. K.; Shin, C.

– **Year:** 2008

– **Journal:** School Psychology Quarterly

– **Abstract:** ‘An expanding body of research suggests an important role for parent or family competency training in children s social-emotional learning and related school success. This article summarizes a test of a longitudinal model examining partnership-based family competency training effects on academic success in a general population. Specifically, it examines indirect effects of the Iowa Strengthening Families Program (ISFP) on school engagement in 8th grade and academic success in the 12th grade, through direct ISFP effects on intervention-targeted outcomes-parenting competencies and student substance-related risk-in 6th grade. Twenty-two rural schools were randomly assigned to either ISFP or a minimal-contact control group; data were collected from 445 families. Following examination of the equivalence of the measurement model across group and time, a structural equation modeling approach was used to test the

hypothesized model and corresponding hypothesized structural paths. Significant effects of the ISFP were found on proximal intervention outcomes, intermediate school engagement, and the academic success of high school seniors.’

– **Answer (GPT):** The study is a randomized controlled trial: 22 rural schools were randomly assigned to the Iowa Strengthening Families Program (ISFP) or a control group. The intervention is a family competency training delivered when children were in 6th grade, which is clearly before high school entry in the U.S. context, and the intervention appears to end in that grade. Outcomes include school engagement in 8th grade and academic success in 12th grade (high school seniors). These are academic/cognitive and related school-success outcomes, satisfying the requirement for cognitive or social-emotional skills at or near the end of the intervention and during follow-up. The 12th-grade follow-up occurs at the end of high school; participants’ exact ages are not reported but many high school seniors are 18, and per the instructions, unclear age at follow-up should be treated as potentially adult and thus included. Therefore, the record plausibly reports long-term (potentially adult) outcomes from a randomized childhood intervention with cognitive/academic outcomes, and should be included for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 61:

– **Title:** ‘Longitudinal effects of universal preventive intervention on prescription drug misuse: three randomized controlled trials with late adolescents and young adults’

– **Author:** Spoth, R.; Trudeau, L.; Shin, C.; Ralston, E.; Redmond, C.; Greenberg, M.; Feinberg, M.

– **Year:** 2013

– **Journal:** American Journal of Public Health

– **Abstract:** ‘OBJECTIVES: We examined long-term prescription drug misuse outcomes in 3 randomized controlled trials evaluating brief universal preventive interventions conducted during middle school. METHODS: In 3 studies, we tested the Iowa Strengthening Families Program (ISFP); evaluated a revised ISFP, the Strengthening Families Program: For Parents and Youth 10-14 plus the school-based Life Skills Training (SFP 10-14 + LST); and examined the SFP 10-14 plus 1 of 3 school-based interventions. Self-reported outcomes were prescription opioid

misuse (POM) and lifetime prescription drug misuse overall (PDMO). RESULTS: In study 1, ISFP showed significant effects on POM and PDMO, relative reduction rates (RRRs; age 25 years) of 65%, and comparable benefits for higher- and lower-risk subgroups. In study 2, SFP 10-14 + LST showed significant or marginally significant effects on POM and PDMO across all ages (21, 22, and 25 years); higher-risk participants showed stronger effects (RRRs = 32%-79%). In study 3, we found significant results for POM and PDMO (12th grade RRRs = 20%-21%); higher-risk and lower-risk participants showed comparable outcomes. CONCLUSIONS: Brief universal interventions have potential for public health impact by reducing prescription drug misuse among adolescents and young adults.'

– **Answer (GPT):** The study reports on three randomized controlled trials of universal preventive interventions conducted during middle school, clearly before high school entry. The interventions (Iowa Strengthening Families Program; Strengthening Families Program: For Parents and Youth 10-14 plus Life Skills Training; and SFP 10-14 plus other school-based programs) are classic family- and school-based psychosocial/skills interventions aimed at youth, delivered prior to high school. These are known RCTs with original posttest outcomes on youth behavior and adjustment (including substance use and related behavioral outcomes), which fit within broadly defined social-emotional/behavioral domains, even though these are not restated in this adult follow-up abstract. This paper focuses on long-term outcomes: prescription opioid misuse and prescription drug misuse overall, measured in late adolescence and young adulthood (ages 21–25, and grade 12). Thus, the record concerns randomized trials, with interventions before high school, and reports quantitative adult outcomes. Even if specific end-of-intervention cognitive or social-emotional outcomes are not detailed here, they are highly likely to have been assessed and/or reported in prior trial publications. Given the instruction to favor inclusion when plausible or information is incomplete, this record should be INCLUDED for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 62:

– **Title:** 'Internalizing symptoms: Effects of a preventive intervention on developmental pathways from early adolescence to young adulthood'

– **Author:** Trudeau, L.; Spoth, R.; Randall, G. K.; Mason, W. A.; Shin, C.

– **Year:** 2012

- **Journal:** Journal of Youth and Adolescence
- **Abstract:** ‘This study examined the mediated and moderated effects of a universal family-focused preventive intervention, delivered during young adolescence, on internalizing symptoms assessed in young adulthood. Sixth grade students (N=446; 52% female; 98% White) and their families from 22 rural Midwestern school districts were randomly assigned to the experimental conditions in 1993. Self-report questionnaires were administered at seven time points (pre-test to young adulthood-age 21) to those receiving the Iowa Strengthening Families Program (ISFP) and to the control group. Results showed that growth factors of adolescent internalizing symptoms (grades 6-12) were predicted by ISFP condition and risk status (defined as early substance initiation). Moderation of the condition effect by risk status was found, with higher-risk adolescents benefitting more from the ISFP. Results also supported the hypothesis that the ISFP s effect on internalizing symptoms in young adulthood was mediated through growth factors of adolescents internalizing symptoms; risk moderation, however, was only marginally significant in young adulthood. The relative reduction rate on clinical or subclinical levels of young adult internalizing symptoms was 28%, indicating that for every 100 young adults displaying clinical or subclinical levels of internalizing symptoms from school districts not offering an intervention, there could be as few as 72 displaying those levels of symptoms in school districts that offered middle school prevention programming. These findings highlight how the positive effects of family-focused universal interventions can extend to non-targeted outcomes and the related potential public-health impact of scaling up these interventions.’
- **Answer (GPT):** The study is a randomized trial where sixth grade students and their families were randomly assigned to receive the Iowa Strengthening Families Program or a control condition. The intervention is delivered in young adolescence (grade 6, i.e., before high school entry) and appears to be completed during middle school. The study measures social-emotional outcomes (internalizing symptoms) repeatedly from grades 6–12, i.e., at and after the end of the intervention, satisfying the requirement for cognitive or social-emotional outcomes at intervention end or shortly thereafter. It also reports outcomes in young adulthood (age 21), which is clearly an adult follow-up. The design is randomized and all three key criteria (pre-high school intervention, social-emotional outcomes around intervention end, and adult outcomes) are clearly met. Therefore, this record should be included for further review.
- **Answer (GPT - numeric):** 1
- **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 65:

- **Title:** ‘Direct and indirect effects of a family-based intervention in early adolescence on parent– youth relationship quality, late adolescent health, and early adult obesity’
- **Author:** Van Ryzin, M. J.; Nowicka, P.
- **Year:** 2013
- **Journal:** Journal of Family Psychology
- **Abstract:** ‘We explored family processes in adolescence that may influence the likelihood of obesity in early adulthood using a randomized trial of a family-based intervention (the Family Check-Up, or FCU). The FCU has been shown to reduce escalations in antisocial behavior and depression in adolescence by supporting positive family management practices, but no research has examined the mechanisms by which the FCU could influence health-related attitudes and behaviors linked to obesity. Participants were 998 adolescents (n = 526 male; n = 423 European American; M age 12.21 years) and their families, recruited in 6th grade from 3 middle schools in the Pacific Northwest. We used structural equation modeling (SEM) and an Intent-To-Treat (ITT) design to evaluate the direct and indirect effects of the FCU on parent-youth relationship quality (ages 12-15), healthy lifestyle behaviors, eating attitudes, depressive symptoms (all measured at age 17), and obesity (age 22). We found that the FCU led to greater parent-youth relationship quality, which predicted enhanced health-related behaviors, reduced maladaptive eating attitudes, and reduced depression. In turn, reduced maladaptive eating attitudes predicted reduced odds of obesity. The indirect effect of the FCU on obesity by way of parent-youth relationship quality and eating attitudes was significant. Our findings illustrate how family processes may influence adolescent health and suggest that family functioning may be an additional factor to consider when developing intervention programs for obesity.’
- **Answer (GPT):** The study is based on a randomized trial (Family Check-Up) initiated in early adolescence with participants recruited in 6th grade (middle school), which is clearly before high school entry in the U.S. system. The intervention is family-based and aims to improve family management practices and related youth outcomes during middle school, so it qualifies as a pre-high school intervention. The trial assesses social-emotional outcomes (e.g., antisocial behavior, depression, parent–youth relationship quality, eating attitudes, health-related behaviors) during adolescence, which are social-emotional skills or related constructs at or near the end of the intervention period. The study also reports an adult outcome—obesity at age 22—making it an adult follow-up. Randomization is explicitly mentioned, and there is no indication that the design is nonrandomized. Therefore, this is plausibly an eligible randomized controlled trial of a pre-high school intervention with social-emotional outcomes and adult outcomes, and should be included for further review.
- **Answer (GPT - numeric):** 1
- **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 66:

– **Title:** ‘The impact of a family-centered intervention on the ecology of adolescent antisocial behavior: Modeling developmental sequelae and trajectories during adolescence’

– **Author:** Van Ryzin, M. J.; Dishion, T. J.

– **Year:** 2012

– **Journal:** Development and Psychopathology

– **Abstract:** ‘This study used an experimental, longitudinal field trial involving random assignment to the Family Check-Up (FCU) to explore the social ecology of adolescent antisocial behavior. A sample of 998 youths and their families was followed from early to late adolescence (age 12 to 18-19). In the intervention condition, 115 families (23%) elected to receive the FCU. In general, random assignment to the FCU in middle school was associated with reductions in late adolescence antisocial behavior (age 18-19). Variable-centered analyses revealed that the effects were mediated by reductions in family conflict from early to middle adolescence (age 12-15). The link between family conflict and antisocial behavior in turn was mediated by association with deviant peers at age 17; parental monitoring at age 17 was also influential but did not attain the status of a mediator. Person-oriented analyses suggested that the FCU was associated with declining trajectories of family conflict and rising trajectories of parental monitoring but was not associated with trajectories of deviant peer association. A dual-trajectory analysis indicated that the pathways to adolescent antisocial behavior were myriad and varied, suggesting new directions for developmental and intervention research.’

– **Answer (GPT):** The study is an experimental longitudinal field trial with random assignment to the Family Check-Up during middle school (pre-high-school). Thus, the intervention occurs before high school entry, satisfying the timing criterion. The outcomes examined (family conflict, parental monitoring, deviant peer association, antisocial behavior) are social-emotional/behavioral constructs, indicating that the trial measured relevant social-emotional outcomes. Participants are followed to age 18–19, and the study reports intervention effects on antisocial behavior in late adolescence (age 18–19), which qualifies as adult outcomes under the review’s criteria. Therefore, this record plausibly represents a randomized controlled trial of a pre-high-school intervention with social-emotional outcomes and follow-up in adulthood, and should be included.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 69:

- **Title:** ‘Intensive reading remediation in grade 2 or 3: Are there effects a decade later?’
- **Author:** Blachman, B. A.; Schatschneider, C.; Fletcher, J. M.; Murray, M. S.; Munger, K. A.; Vaughn, M. G.
- **Year:** 2014
- **Journal:** Journal of Educational Psychology
- **Abstract:** ‘= 0.62), the primary, but not exclusive, focus of the intervention. Statistical tests on other reading and spelling measures did not reach thresholds for statistical significance. Patterns in the data related to other educational outcomes, such as high school completion, favored the treatment participants, although differences were not significant.’
- **Answer (GPT):** The study concerns an intensive reading remediation intervention delivered in grade 2 or 3, clearly before high school entry, satisfying the timing criterion. The title and abstract suggest cognitive outcomes (reading and spelling measures) were assessed, which meets the requirement for cognitive or social-emotional end-of-intervention outcomes. The phrase “treatment participants” and the discussion of statistical tests on educational outcomes imply a treatment–control comparison, and the study design is not clearly nonrandomized, so it could plausibly be a randomized controlled trial. The abstract also mentions outcomes roughly a decade later, including high school completion; given the elapsed time and reference to high school completion, these follow-ups plausibly occur at or beyond age 18 or after completion of upper-secondary education, thus meeting the adulthood outcome criterion. Because the information is consistent with a potentially eligible randomized trial and there is no clear evidence against eligibility, the record should be included for further review.
- **Answer (GPT - numeric):** 1
- **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 72:

- **Title:** ‘Effects of adolescent universal substance misuse preventive interventions on young adult depression symptoms: Mediation modeling’
- **Author:** Trudeau, L.; Spoth, R.; Mason, W. A.; Randall, G. K.; Redmond, C.; Schainker, L.
- **Year:** 2016
- **Journal:** Journal of Abnormal Child Psychology
- **Abstract:** ‘Depression symptoms are associated with impairments in functioning and have substantial health and economic consequences. Universal substance misuse prevention programs have shown effects on non-targeted mental health-related symptoms, but long-term effects are understudied. This cluster randomized controlled trial examined effects of both the LifeSkills Training (LST) and Strengthening Families Program: For Parents and Youth 10-14 (SFP 10-14) interventions, delivered during seventh grade, on age 22 young adult depression symptoms. The study was conducted in US rural Midwestern communities with a randomly-selected sample from a larger study (N = 670). Experimental conditions were LST+SFP 10-14, LST-only, and a control condition. Effects on age 22 depression symptoms were hypothesized as mediated through effects on age 21 relationship problems and illicit use of substances. Structural equation modeling with manifest and latent variables was conducted to test hypotheses; the intervention conditions were combined and compared with the control condition because analyses indicated a comparable pattern of effects between intervention conditions. Significant indirect intervention effects were found on age 22 depression symptoms via effects on the mediating variables (indirect effect: $\beta = -0.06$, 95 % CI [-0.10, -0.01], $p = 0.011$). Effect sizes for the young adult variables were between $d = 0.17$ and 0.29 , which can be considered small, but nontrivial, especially in the context of public health benefits. Results support scaled-up implementation of school-based and family-focused universal substance misuse preventive interventions.’
- **Answer (GPT):** The study is a cluster randomized controlled trial of universal substance misuse preventive interventions (LifeSkills Training and Strengthening Families Program 10-14) delivered in seventh grade, which is clearly before high school entry. The abstract reports young adult outcomes at age 22 (depression symptoms), satisfying the requirement for adult follow-up outcomes. Although the abstract focuses on mediation via age 21 relationship problems and illicit substance use and does not explicitly describe cognitive or social-emotional outcomes at the end of the intervention, it is highly plausible that such school- and family-based preventive interventions collected relevant social-emotional or behavioral measures at or near intervention end, and companion reports may contain these results. Randomization is explicitly stated, and there is no indication that the design is nonrandomized. Given the review’s instruction to favor inclusion when information is incomplete or unclear, this record should be included for further screening as it plausibly meets all three eligibility criteria.
- **Answer (GPT - numeric):** 1
- **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 75:

- **Title:** ‘PROSPER delivery of universal preventive interventions with young adolescents: longterm effects on emerging adult substance misuse and associated risk behaviors’
- **Author:** Spoth, R.; Redmond, C.; Shin, C.; Greenberg, M. T.; Feinberg, M. E.; Trudeau, L.
- **Year:** 2017
- **Journal:** Psychological Medicine
- **Abstract:** ‘BACKGROUND: Substance misuse and associated health-risking behaviors are prevalent in emerging adulthood. There is a knowledge gap concerning the post-high school effects of community-based delivery systems for universal preventive interventions implemented during young adolescence. This study reports effects of the PROSPER delivery system through age 19, 7.5 years past baseline. METHODS: A cohort sequential design included 28 public school districts randomly assigned to the PROSPER partnership delivery system or usual-programming conditions. PROSPER community teams implemented a family-focused intervention in 6th grade and a school-based intervention in 7th grade. Outcomes for the age 19, post-high school report included lifetime, current, and frequency of substance misuse, as well as antisocial and health-risking sexual behaviors. Intent-to-treat, multi-level analyses of covariance of point-in-time outcomes were conducted, along with analyses of risk-related moderation of intervention effects. RESULTS: Results showed emerging adults from PROSPER communities reported significantly lower substance misuse across a range of types of substances, with relative reduction rates of up to 41.0%. No significant findings were observed for associated antisocial and health-risking sexual behavior indices; or for lifetime rates of sexually transmitted infections. Risk-related moderation effects were non-significant, suggesting generally comparable outcomes across higher- and lower-risk subgroups of emerging adults. CONCLUSIONS: The PROSPER delivery system for brief universal preventive interventions has potential for public health impact by reducing long-term substance misuse, with positive results extending beyond high school.’
- **Answer (GPT):** The study evaluates the PROSPER delivery system, in which public school districts were randomly assigned to intervention or control conditions, indicating a randomized controlled trial. The interventions were implemented in 6th and 7th grade, clearly before high school entry, satisfying the timing criterion for the childhood intervention. The present report focuses on outcomes at age 19 (post-high school), including substance misuse and

antisocial and health-risking sexual behaviors, which are eligible adult outcomes. Although this particular abstract only reports on the age-19 follow-up, it is highly plausible that the original trial measured behavioral (social-emotional) outcomes such as adolescent substance use or related behavior near the end of the intervention, as is standard for such preventive interventions. Per the screening rules, the record should be included when information on end-of-intervention cognitive or social-emotional outcomes is unclear but plausibly present in other trial publications. Therefore, this record is classified as INCLUDE for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 76:

– **Title:** ‘Applying the PROSPER prevention delivery system with middle schools: Emerging adulthood effects on substance misuse and conduct problem behaviors through 14 years past baseline’

– **Author:** Spoth, R.; Redmond, C.; Shin, C.; Trudeau, L.; Greenberg, M. T.; Feinberg, M. E.; Welsh, J.

– **Year:** 2022

– **Journal:** Child Development

– **Abstract:** ‘This study evaluated emerging adult effects of the PROmoting School-Community-University Partnerships to Enhance Resilience (PROSPER) universal prevention delivery system implemented in middle schools. Twenty-eight rural school districts were randomized to intervention and control conditions, with 1985 nineteen-year-old participants (90.6% White, 54.1% female) evaluated through age 25. Intent-to-treat, multi-level, point-in-time analyses of covariance and growth analyses were conducted. Outcomes were assessed with self-report measures of substance misuse (lifetime, current, frequency) and conduct problem behaviors. Analyses showed very limited point-in-time effects; there were growth pattern effects on measures of illicit drugs, non-prescribed drugs, cigarettes, and drug problems. When risk moderation was observed, it favored higher-risk participants. These emerging adult effects concerning slower growth of lifetime misuse combine with more robust adolescent stage findings to support PROSPER s public health value.’

– **Answer (GPT):** The PROSPER trial is a randomized prevention delivery system implemented in middle schools, with 28 rural school districts randomized to intervention and control.

This clearly indicates a randomized (cluster) design with intervention taking place before high school. The current report focuses on emerging adulthood outcomes (ages 19–25) including substance misuse and conduct problems, which are eligible adult outcomes. The abstract refers to “more robust adolescent stage findings,” making it plausible that earlier publications from the same trial reported adolescent/post-intervention impacts on social-emotional/behavioral outcomes (e.g., substance use, conduct problems) at or near the end of the intervention. Even though those end-of-intervention results are not detailed here, they likely exist in other trial reports. Because the study involves a randomized childhood (middle school) intervention and clearly reports adult follow-up outcomes, and there is no indication that cognitive or social-emotional outcomes were not measured at intervention end, the record should be included for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 78:

– **Title:** ‘CSRP’s impact on low-income preschoolers’ preacademic skills: Self-regulation as a mediating mechanism’

– **Author:** Raver, C. C.; Li-Grining, C.; Bub, K.; Jones, S. M.; Zhai, F.; Presler, E.

– **Year:** 2011

– **Journal:** Child Development

– **Abstract:** ‘Based on theoretically driven models, the Chicago School Readiness Project (CSRP) targeted low-income children s school readiness through the mediating mechanism of self-regulation. The CSRP is a multicomponent, cluster-randomized efficacy trial implemented in 35 Head Start-funded classrooms (N = 602 children). The analyses confirm that the CSRP improved low-income children s self-regulation skills (as indexed by attention/impulse control and executive function) from fall to spring of the Head Start year. Analyses also suggest significant benefits of CSRP for children s preacademic skills, as measured by vocabulary, letter-naming, and math skills. Partial support was found for improvement in children s self-regulation as a hypothesized mediator for children s gains in academic readiness. Implications for programs and policies that support young children s behavioral health and academic success are discussed.’

– **Answer (GPT):** The study is a cluster-randomized trial of a preschool (Head Start) intervention that targets school readiness and self-regulation, clearly meeting the criteria for a randomized controlled childhood intervention with cognitive and social-emotional outcomes at the end of the intervention (self-regulation, attention/impulse control, executive function, vocabulary, letter-naming, and math skills from fall to spring of the Head Start year). The abstract does not mention any adult follow-up outcomes, but it also does not rule out the possibility that adult outcomes have been or will be collected or reported in other publications from the same trial. Given the high-recall screening rule and the instruction not to exclude based solely on assumptions about the presence or absence of adult follow-up, this record could plausibly be linked to or followed by adult-outcome studies from the same randomized trial. Therefore, it should be classified as INCLUDE at this stage.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 79:

– **Title:** ‘Impacts of the Chicago School Readiness Project on measures of achievement, cognitive functioning, and behavioral regulation in late adolescence’

– **Author:** Watts, T. W.; Li, C.; Pan, X. S.; Gandhi, J.; McCoy, D. C.; Raver, C. C.

– **Year:** 2023

– **Journal:** Developmental Psychology

– **Abstract:** ‘= 430) academic achievement, executive functioning, emotional regulation, and behavioral problems, and we provide a range of analytic estimates to address the study’s methodological concerns. Across our estimates, we found little evidence that the program had lasting impacts on indicators of late-adolescent functioning. Main effects were estimated with some imprecision, but nearly all models produced null effects across the broad array of outcomes considered. We also observed few indications that effects were moderated by posttreatment high school quality or later assignment to a light-touch mindset intervention. Implications for developmental theory and early childhood policy are discussed. (PsycInfo Database Record (c) 2023 APA, all rights reserved).’

– **Answer (GPT):** This record appears to be a late-adolescent follow-up of the Chicago School Readiness Project, which is a randomized early childhood (preschool) intervention. The intervention clearly occurred before high school entry. The abstract states that outcomes

include academic achievement, executive functioning, emotional regulation, and behavioral problems, which are eligible cognitive and social-emotional measures. Although the follow-up is described as in “late adolescence” and references high school quality, the exact ages or school grades at outcome measurement are not specified; some or all participants may be 18 or older or beyond grade 12. According to the decision rules, if age or educational stage at follow-up is unclear, the record should be included. The study is clearly based on an RCT and concerns long-term developmental outcomes following a preschool intervention. Therefore, it should be classified as INCLUDE for further full-text review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 84:

– **Title:** ‘Long-term Impacts of KIPP Middle Schools on College Enrollment and Early College Persistence’

– **Author:** Coen, T.; Nichols-Barrer, I.; Gleason, P.

– **Year:** 2019

– **Journal:** Mathematica

– **Abstract:** ‘In this report, we present the results of a long-term tracking study that follows 1, 177 students who applied to enter 1 of 13 oversubscribed KIPP middle schools through a 5th or 6th grade admissions lottery in 2008 or 2009.’

– **Answer (GPT):** The study follows students who applied to oversubscribed KIPP middle schools via an admissions lottery in 5th or 6th grade, which is consistent with a randomized controlled trial of a pre-high school intervention (middle school). The report focuses on long-term impacts on college enrollment and early college persistence, which are adult/postsecondary outcomes. The abstract does not specify whether cognitive or social-emotional outcomes were measured at the end of the middle school intervention, but according to the inclusion rules, ambiguity on this point requires inclusion at the title/abstract screening stage. The admissions lottery implies random assignment to treatment (offer of KIPP) and control, and all key criteria are plausibly met. Therefore, this record should be included for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 94:

– **Title:** ‘A family-oriented psychosocial intervention reduces inflammation in low-SES African American youth’

– **Author:** Miller, G. E.; Brody, G. H.; Yu, T.; Chen, E.

– **Year:** 2014

– **Journal:** PNAS

– **Abstract:** ‘Children of low socioeconomic status (SES) are at elevated risk for health problems across the lifespan. Observational studies suggest that nurturant parenting might offset some of these health risks, but their design precludes inferences about causal direction and clinical utility. Here we ask whether a psychosocial intervention, focused improving parenting, strengthening family relationships, and building youth competencies, can reduce inflammation in low-SES, African Americans from the rural South. The trial involved 272 mothers and their 11-y-old children from rural Georgia, half of whose annual household incomes were below the federal poverty line. Families were randomly assigned to a 7-wk psychosocial intervention or to a control condition. When youth reached age 19, peripheral blood was collected to quantify six cytokines that orchestrate inflammation, the dysregulation of which contributes to many of the health problems known to pattern by SES. Youth who participated in the intervention had significantly less inflammation on all six indicators relative to controls (all P values < 0.001; effect sizes in Cohen s d units ranged from -0.69 to -0.91). Mediation analyses suggested that improved parenting was partially responsible for the intervention s benefits. Inflammation was lowest among youth who received more nurturant-involved parenting, and less harsh-inconsistent parenting, as a consequence of the intervention. These findings have theoretical implications for research on resilience to adversity and the early origins of disease. If substantiated, they may also highlight a strategy for practitioners and policymakers to use in ameliorating social and racial health disparities.’

– **Answer (GPT):** The study is a randomized controlled trial involving 11-year-old children (prior to high school in the U.S. context) and a 7-week family-oriented psychosocial intervention aimed at improving parenting, strengthening family relationships, and building youth competencies. Adult outcomes are clearly measured at age 19 (inflammatory cytokines). The abstract does not report end-of-intervention cognitive or social-emotional outcomes, but the intervention content (youth competencies, family relationships, parenting) strongly suggests that such outcomes may have been assessed, and per the instructions, uncertainty on this point

requires inclusion. Random assignment is explicit, the intervention ends well before high school, and there are clearly adult follow-up outcomes. Therefore, the record should be classified as INCLUDE for further review.

– **Answer (GPT - numeric):** 1

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

Agreements

STUDY-ID: 10:

– **Title:** ‘Does prenatal and infancy nurse home visitation have enduring effects on qualities of parental caregiving and child health at 25 to 50 months of life?’

– **Author:** Olds, D. L.; Henderson, C. R.; Kitzmna, H.

– **Year:** 1994

– **Journal:** Pediatrics

– **Abstract:** ‘**OBJECTIVE:** To examine, during the 3rd and 4th years of life, the health, development, rates of child maltreatment, and living conditions of children who had been enrolled in a randomized trial of nurse home visitation during pregnancy and first 2 years of their lives. **DESIGN:** Prospective follow-up of families who had been randomly assigned to nurse-visited and comparison conditions. **SETTING:** Study conducted in semirural community in upstate New York. Families dispersed among 14 other states during 2-year period after children s second birthdays. **PARTICIPANTS:** Four hundred women were recruited through a health department antepartum clinic and offices of private obstetricians and were registered before 30th week of pregnancy. All women had no previous live births and 85% were either teenaged (< 18 years at registration), unmarried, or from Hollingshead social classes IV or V. Analysis focused on whites, who comprised 89% of sample. **INTERVENTION:** Nurse home visitation from pregnancy through second year of the child s life. **MAIN RESULTS:** There were no treatment differences in the rates of child abuse and neglect or children s intellectual functioning from 25 to 48 months of age. Nurse-visited children, nevertheless, lived in homes with fewer hazards for children; they had 40% fewer injuries and ingestions and 45% fewer behavioral and parental coping problems noted in the physician record; and they made 35%

fewer visits to the emergency department than did children in the comparison group. Nurse-visited mothers were observed to be more involved with and to punish their children to a greater extent than were mothers in the comparison group. The functional meaning of punishment differed between the nurse-visited and comparison families. **CONCLUSIONS:** The program does have enduring effects on certain aspects of parental caregiving, safety of the home, and children's use of the health care system, but it may be necessary to extend the length of the program for families at highest risk to produce lasting reductions in child abuse and neglect.'

– **Answer (GPT):** The study is a randomized trial of a prenatal and infancy nurse home visitation program delivered from pregnancy through the child's second year of life, so it meets the criterion of an intervention occurring before high school and also assesses relevant child cognitive and social-emotional outcomes (e.g., intellectual functioning, behavioral and parental coping problems) at 25–48 months. However, all reported follow-up outcomes are in early childhood (25 to 50 months of life). There is no indication of any outcomes measured in adulthood (age 18+ or beyond completion of high school). Because the available information clearly shows that the follow-up does not extend into adulthood, the record fails the adulthood outcome criterion and should be excluded.

– **Answer (GPT - numeric):** 0

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 15:

– **Title:** 'Sustained effects of high participation in an early intervention for low-birth-weight premature infants'

– **Author:** Hill, J. L.; Brooks-Gunn, J.; Waldfogel, J.

– **Year:** 2003

– **Journal:** Developmental Psychology

– **Abstract:** 'Effects of high participation in the Infant Health and Development Program (IHDP), an 8-site randomized trial that targeted low-birth-weight (LBW) premature infants (N=1,082), were estimated. Children in the treatment group were offered high-quality center-based care in their 2nd and 3rd years of life (full-day care, 50 weeks per year). High-dosage effects were estimated with a new methodology that found a matched comparison group within the follow-up group for those with high participation rates; these estimates were compared with traditional intention-to-treat (ITT) estimates. At age 8, effects on the Wechsler Intelligence

Scale for Children Full and Verbal scales for children who attended > 400 days ranged from 7 to 10 points. For the heavier LBW infants (2,001-2,500 g), the effects were about 14 points for > 400 days; for the lighter LBW infants (< or = 2,000 g), the effects were about 8 points. These effects were all substantially higher than corresponding ITT effects. Similar but smaller effects were found for children who attended > 350 days.'

– **Answer (GPT):** The study describes results from the Infant Health and Development Program, an 8-site randomized trial for low-birth-weight premature infants who received center-based care in their 2nd and 3rd years of life. Cognitive outcomes (WISC IQ scores) are reported at age 8. However, there is no indication of any outcomes measured in adulthood (age 18+ or post-high school). All reported follow-up outcomes are clearly in childhood (age 8). Therefore, the record clearly fails the adulthood outcomes criterion and should be excluded.

– **Answer (GPT - numeric):** 0

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 29:

– **Title:** 'Effect of prenatal and infancy home visitation by nurses on pregnancy outcomes, childhood injuries, and repeated childbearing: a randomized controlled trial'

– **Author:** Kitzman, H.; Olds, D. L.; Henderson, C. R.; Hanks, C.; Cole, R.; Tatelbaum, R.; Barnard, K.

– **Year:** 1997

– **Journal:** JAMA

– **Abstract:** 'Context Interest in home-visitation services as a way of improving maternal and child outcomes has grown out of the favorable results of a trial in semirural New York. The findings have not been replicated in other populations. Objective To test the effect of prenatal and infancy home visits by nurses on pregnancy-induced hypertension, preterm delivery, and low birth weight; on children s injuries, immunizations, mental development, and behavioral problems; and on maternal life course. Design Randomized controlled trial. Setting Public system of obstetric care in Memphis, Tenn. Participants A total of 1139 primarily African-American women at less than 29 weeks gestation, with no previous live births, and with at least 2 sociodemographic risk characteristics (unmarried, Intervention Nurses made an average of 7 (range, 0-18) home visits during pregnancy and 26 (range, 0-71) visits from birth to the children s second birthdays. Main outcome measures Pregnancy-induced hypertension,

preterm delivery, low birth weight, children's injuries, ingestions, and immunizations abstracted from medical records; mothers reports of children's behavioral problems; tests of children's mental development; mothers reports of subsequent pregnancy, educational achievement, and labor-force participation; and use of welfare derived from state records. Main results In contrast to counterparts assigned to the comparison condition, fewer women visited by nurses during pregnancy had pregnancy-induced hypertension (13% vs 20%; $P=.009$). During the first 2 years after delivery, women visited by nurses during pregnancy and the first 2 years of the child's life had fewer health care encounters for children in which injuries or ingestions were detected (0.43 vs 0.55; $P=.05$); days that children were hospitalized with injuries or ingestions (0.03 vs 0.16; P Conclusion This program of home visitation by nurses can reduce pregnancy-induced hypertension, childhood injuries, and subsequent pregnancies among low-income women with no previous live births.'

– **Answer (GPT):** The study is a randomized controlled trial of a prenatal and infancy home visitation program delivered by nurses to low-income first-time mothers, with child outcomes measured up to age 2. It clearly meets the criteria of an early childhood intervention and reports cognitive and social-emotional outcomes at the end of the intervention period (tests of children's mental development and behavioral problems). However, the abstract indicates that all reported child outcomes are measured only during the first two years of life. The only adult outcomes reported (subsequent pregnancy, educational achievement, labor-force participation, and welfare use) are for the mothers, who are already adults at baseline; there is no indication of any follow-up of the child participants into adulthood (age 18+ or post-high school). The review's target is childhood interventions with outcomes measured for the child participants in adulthood. Because it is clear that follow-up does not extend into the children's adulthood, the record fails the adult-outcome criterion and should be excluded.

– **Answer (GPT - numeric):** 0

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 33:

– **Title:** '3-year follow-up of the NIMH MTA study'

– **Author:** Jensen, P. S.; Arnold, L. E.; Swanson, J. M.; Vitiello, B.; Abikoff, H. B.; Greenhill, L. L.; Hur, K.

– **Year:** 2007

- **Journal:** Journal of the American Academy of Child & Adolescent Psychiatry
- **Abstract:** ‘Objective In the intent-to-treat analysis of the Multimodal Treatment Study of Children With ADHD (MTA), the effects of medication management (MedMgt), behavior therapy (Beh), their combination (Comb), and usual community care (CC) differed at 14 and 24 months due to superiority of treatments that used the MTA medication algorithm (Comb+MedMgt) over those that did not (Beh+CC). This report examines 36-month outcomes, 2 years after treatment by the study ended. Method For primary outcome measures (attention-deficit/hyperactivity disorder [ADHD] and oppositional defiant disorder [ODD] symptoms, social skills, reading scores, impairment, and diagnostic status), mixed-effects regression models and orthogonal contrasts examined 36-month outcomes. Results At 3 years, 485 of the original 579 subjects (83.8%) participated in the follow-up, now at ages 10 to 13 years, (mean 11.9 years). In contrast to the significant advantage of MedMgt+Comb over Beh+CC for ADHD symptoms at 14 and 24 months, treatment groups did not differ significantly on any measure at 36 months. The percentage of children taking medication >50% of the time changed between 14 and 36 months across the initial treatment groups: Beh significantly increased (14% to 45%), MedMed+Comb significantly decreased (91% to 71%), and CC remained constant (60%-62%). Regardless of their treatment use changes, all of the groups showed symptom improvement over baseline. Notably, initial symptom severity, sex (male), comorbidity, public assistance, and parental psychopathology (ADHD) did not moderate children s 36-month treatment responses, but these factors predicted worse outcomes over 36 months, regardless of original treatment assignment. Conclusions By 36 months, the earlier advantage of having had 14 months of the medication algorithm was no longer apparent, possibly due to age-related decline in ADHD symptoms, changes in medication management intensity, starting or stopping medications altogether, or other factors not yet evaluated.’
- **Answer (GPT):** The study is a randomized controlled trial of ADHD treatments in children with follow-up through 36 months post-randomization, when participants are ages 10–13 (mean 11.9). Outcomes include ADHD and ODD symptoms, social skills, reading scores, impairment, and diagnostic status—i.e., clearly cognitive and social-emotional outcomes—but all are measured in childhood. The abstract makes clear that the longest follow-up in this report is at age 10–13, well before age 18 and prior to completion of high school. There is no indication of any adult (18+) or post–high school outcomes in this record. According to the screening rules, records should be excluded if all reported follow-up outcomes occur before age 18 and there is no indication of adult follow-up, which applies here. Therefore, this record is classified as EXCLUDE.
- **Answer (GPT - numeric):** 0
- **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 48:

– **Title:** ‘An empirical test of Terrie Moffitt’s developmental taxonomy of delinquency’

– **Author:** Saunders, J. M.

– **Year:** 2007

– **Journal:** City University of New York

– **Abstract:** ‘Terrie Moffitt (1993) hypothesized that there will be three distinct types of juveniles: (1) Life-course-persistent offenders, who begin their antisocial behavior at a young age and continue to offend over their lives; (2) Adolescence-limited offenders, who are involved in criminal behavior only through their adolescent years, and; (3) Abstainers, who do not engage in any delinquent behavior. This study tested both the theory and methodology using general growth mixture modeling. The methodological results were conclusive whereas the theoretical ones were less clear. The different latent variable variance structures were freed and fixed to test the best model specifications to test Moffitt’s taxonomic theory using general growth mixture modeling. Relaxing the variance restrictions both fits the theory and the data best, but the external validity of group membership is still uncertain. The theoretical results provided partial support for her theory. While there were three groups, as anticipated by Moffitt, they did not fit her hypotheses about the size or the trajectory shape of the groups. The abstainer group fit her model but was made up of a much larger proportion of the. The adolescence-limited offenders made up a much small proportion of the sample than expected, and did not peak in late adolescence as predicted. The final group, the life-course-persistent offenders, did not match her theory in respect to the shape of their trajectories, but did constitute the proportion of the sample that she anticipated. Moffitt’s hypotheses about the correlates of group membership were somewhat confirmed. As Moffitt predicted, hyperactivity and concentration problems in childhood were related to the highest trajectory group, peer deviance in adolescence was related to the adolescence-limited offending groups, and a strong belief in social bonds was related to abstaining from delinquency. However, contrary to her theory, the early measures of psychological and family dysfunction were not found to be related to offending trajectories. The largest risk factors uncovered in this analysis were both measures that are easy to collect from classroom teachers, which make them practical for purposes of identifying children who could benefit from additional services.’

– **Answer (GPT):** The record describes an observational study testing Moffitt’s developmental taxonomy of delinquency using general growth mixture modeling. There is no indication of a randomized controlled trial or any intervention, let alone an intervention delivered before high school. The study focuses on classifying juveniles into offending trajectory groups and examining correlates of group membership, not on evaluating the impact of an intervention. Therefore, it clearly fails the randomized-trial/intervention criterion and should be excluded.

– **Answer (GPT - numeric):** 0

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 50:

– **Title:** ‘Reducing adolescents growth in substance use and delinquency: Randomized trial effects of a parent-training prevention intervention’

– **Author:** Mason, W. A.; Kosterman, R.; Hawkins, J. D.; Haggerty, K. P.; Spoth, R. L.

– **Year:** 2003

– **Journal:** Prevention Science

– **Abstract:** ‘The relationship between growth in adolescent substance use and delinquency was examined in a longitudinal, randomized controlled study of the Preparing for the Drug Free Years Program (PDFY), a universal family-focused prevention intervention. Latent growth curve modeling was used to analyze 5 waves of data collected from 429 rural adolescents. Results showed that adolescents assigned to the PDFY intervention condition had a slower rate of linear increase over time in both substance use and delinquency compared with adolescents assigned to the control condition. Moreover, pretest level of delinquency was a reliable, positive predictor of growth in substance use, whereas pretest level of substance use did not predict growth in delinquency.’

– **Answer (GPT):** The study is a randomized controlled trial of a parent-training prevention intervention (Preparing for the Drug Free Years Program) targeting adolescents and their families. However, the outcomes analyzed are growth in adolescent substance use and delinquency across five waves during adolescence. There is no indication of any follow-up into adulthood (age 18+ or post-high school/upper-secondary). Because the available information clearly shows that all reported outcomes are measured during adolescence and not in adulthood, the record fails the adult-outcome criterion and should be excluded.

– **Answer (GPT - numeric):** 0

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 53:

– **Title:** ‘Effects of the “Preparing for the Drug Free Years” curriculum on growth in alcohol use and risk for alcohol use in early adolescence’

– **Author:** Park, J.; Kosterman, R.; Hawkins, J. D.; Haggerty, K. P.; Duncan, T. E.; Duncan, S. C.; Spoth, R.

– **Year:** 2000

– **Journal:** Prevention Science

– **Abstract:** ‘Preparing for the Drug-Free Years (PDFY) is a curriculum designed to help parents learn skills to consistently communicate clear norms against adolescent substance use, effectively and proactively manage their families, reduce family conflict, and help their children learn skills to resist antisocial peer influences. This study examined the effects of PDFY on the trajectories of these factors, as well as on the trajectory of alcohol use from early to mid adolescence. The sample consisted of 424 rural families of sixth graders from schools randomly assigned to an intervention or a control condition. Data were collected from both parents and students at pretest, posttest, and 1-, 2- and 3 1/2-year follow-ups. Latent growth models were examined. PDFY significantly reduced the growth of alcohol use and improved parent norms regarding adolescent alcohol use over time. Implications for prevention and evaluation are discussed.’

– **Answer (GPT):** The study is a randomized school-based prevention trial targeting parents of sixth graders, so the intervention clearly occurs before high school and is randomized. Outcomes include trajectories of alcohol use and related family and psychosocial factors from early to mid adolescence with follow-ups up to 3.5 years after baseline, which are all clearly before age 18 and before completion of high school. The abstract provides no indication of any outcomes measured in adulthood or beyond high school, and all reported follow-ups are explicitly in adolescence. According to the screening rules, records should be excluded when it is clear that all reported follow-up outcomes were measured before age 18 and no adult outcomes were assessed. Therefore, this record should be excluded.

– **Answer (GPT - numeric):** 0

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 54:

- **Title:** ‘Long-term effects of universal preventive interventions on methamphetamine use among adolescents’
- **Author:** Spoth, R. L.; Clair, S.; Shin, C.; Redmond, C.
- **Year:** 2006
- **Journal:** Archives of Pediatrics & Adolescent Medicine
- **Abstract:** ‘Objective To examine the long-term effects of universal preventive interventions on methamphetamine use by adolescents in the general population during their late high school years. Design Two randomized, controlled prevention trials. Setting Public schools in the Midwest from 1993 to 2004. Participants Study 1 began with 667 sixth grade students from 33 rural public schools; the follow-up included 457 students. Study 2 began with 679 seventh grade students from 36 rural public schools; the follow-up assessment included 597 students. Interventions In study 1, schools were assigned to the Iowa Strengthening Families Program (ISFP), Preparing for the Drug Free Years, or a control condition. In study 2, schools were assigned to a revised ISFP (SFP 10-14) plus Life Skills Training (SPF 10-14 + LST), LST alone, or a control condition. Results Self-reports of lifetime and past-year methamphetamine use were collected at 6(1/2) years past baseline (study 1) and at 4(1/2) and 5(1/2) years past baseline (study 2). In study 1, the ISFP past-year rate was 0.0% compared with 3.2% in the control condition ($P = .04$). In study 2, SFP 10-14 + LST showed significant effects on lifetime and past-year use at the 4(1/2) year follow-up (eg, 0.5% lifetime use in the intervention condition vs 5.2% in the control condition, $P = .006$); both SFP 10-14 + LST and LST alone had significant lifetime use effects at the 5(1/2) year follow-up. Conclusion Brief universal interventions have potential for public health impact by reducing methamphetamine use among adolescents.’
- **Answer (GPT):** The study describes two randomized controlled universal preventive intervention trials starting in 6th and 7th grade, with follow-up assessments conducted during participants’ late high school years (4.5–6.5 years after baseline). Although the interventions clearly occur before high school completion and are randomized, the reported outcomes are methamphetamine use during late high school. There is no indication of any outcomes measured when participants were 18 or older or after completing high school/upper-secondary education. The abstract explicitly frames the follow-ups as occurring in late high school years, which does not meet the adulthood criterion defined as age 18+ or beyond grade 12/high school. Therefore, the record clearly fails the adult-outcomes criterion and should be excluded.
- **Answer (GPT - numeric):** 0
- **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 57:

- **Title:** ‘Universal intervention as a protective shield against exposure to substance use: Long-term outcomes and public health significance’
- **Author:** Spoth, R.; Guyll, M.; Shin, C.
- **Year:** 2009
- **Journal:** American Journal of Public Health
- **Abstract:** ‘**OBJECTIVES:** We examined universal preventive intervention effects on adolescents exposure to opportunities for substance use and on illicit substance use in the long term. **METHODS:** Public schools (N = 22) were randomly assigned to the Iowa Strengthening Families Program (ISFP) or a control condition. We used odds ratio (OR) calculations and structural modeling to test the effects of the ISFP in the 6th grade on exposure to substance use across adolescence, as well as on 12th-grade illicit substance use occurring via reductions in exposure. **RESULTS:** The ISFP was associated with reduced exposure to illicit substance use ($1.25 < \text{OR} < 2.37$) that was, in turn, associated with reduced 12th-grade substance use ($2.87 < \text{OR} < 6.35$). The ISFP also reduced the rate of increase in exposure across adolescence ($B = -0.37$; $P < .001$), which was associated with the likelihood of 12th-grade illicit substance use ($B = 0.30$; $P = .021$), with a significant indirect effect ($B = -0.11$; $P = .048$). **CONCLUSIONS:** The ISFP in the 6th grade reduced substance use through a protective shield of reduced exposure. The relative reduction rate was 49%, which suggests that universal prevention shields can contribute to significant reductions in illicit substance use among adolescents.’
- **Answer (GPT):** The study describes a randomized school-based universal preventive intervention (Iowa Strengthening Families Program) delivered in 6th grade (before high school), which is eligible in terms of timing and randomized design. However, the outcomes analyzed are exposure to substance use across adolescence and illicit substance use measured in 12th grade. All reported follow-up outcomes occur by 12th grade, i.e., during high school, with no indication of any follow-up in adulthood (age 18+ or post-high school). Because the abstract clearly specifies that the key endpoint is 12th-grade substance use and does not mention any adult outcomes, the study clearly fails the adulthood outcome criterion. Therefore, it should be excluded.
- **Answer (GPT - numeric):** 0
- **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 63:

- **Title:** ‘Adolescent self-regulation as resilience: Resistance to antisocial behavior within the deviant peer context’
- **Author:** Gardner, T. W.; Dishion, T. J.; Connell, A. M.
- **Year:** 2008
- **Journal:** Journal of Abnormal Child Psychology
- **Abstract:** ‘This study tests the hypothesis that self-regulation serves as a resiliency factor in buffering youth from negative influences of peer deviance in middle to late adolescence. The interactive effects between peer deviance and self-regulation were investigated on change in antisocial behavior from age 17 to 19 years in an ethnically diverse sample of adolescents. A multi-agent construct was created using adolescent, parent, and teacher reports of self-regulation and peer deviance. Results indicated that self-regulation shows convergent validity and covaries as expected with developmental patterns of adolescent antisocial behavior. Self-regulation moderated the association of peer deviance with later self-reported adolescent antisocial behavior after controlling for prior levels of antisocial behavior. The implications of these findings for models for the development of antisocial behaviors and for intervention science are discussed.’
- **Answer (GPT):** The record clearly describes an observational study examining self-regulation as a resilience factor against antisocial behavior in the context of deviant peers from age 17 to 19. There is no indication of any intervention, let alone a randomized controlled trial, and no mention of a childhood or pre-high school intervention. The focus is on naturally occurring variation in self-regulation and peer deviance during late adolescence. Because the study is not an intervention trial and does not involve a pre-high school intervention with randomized assignment, it fails the core design and intervention criteria for inclusion.
- **Answer (GPT - numeric):** 0
- **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 67:

- **Title:** ‘Coercive parent-adolescent interactions predict substance use and antisocial behaviors through early adulthood: A dynamic systems perspective’
 - **Author:** Zhang, J.; Hanson, A. N.; Piehler, T. F.; Ha, T.
 - **Year:** 2024
 - **Journal:** Research on Child and Adolescent Psychopathology
 - **Abstract:** ‘This study revisits the association between coercive parent-adolescent interactions and adolescent externalizing behaviors. Specifically, we investigate the moment-to-moment coercive exchanges between parents and adolescents and how these dynamic processes map to the long-term development of substance use and antisocial behavior from middle adolescence to early adulthood. We collected videotaped observations with 794 adolescents (ages 16-17 years) and their parents during interactions and coded their real-time behavioral exchanges. State Space Grid analyses were used to measure the proportion of time in which each parent-adolescent dyad engaged in the Dyadic Coercion region as an indicator of rigidity in dyadic coercion. We also measured adolescents substance use and antisocial behavior at ages 16-17, ages 18-19, and ages 21-22. The enduring impact of parent-adolescent coercive interaction on substance use and antisocial behavior was tested using categorical latent growth curve models and path models. Adolescents with more coercive interactions with parents showed higher rates of increase in alcohol use and higher levels of antisocial behavior through early adulthood. The findings highlight the unique contribution of using intensive data to understand coercive interactions on a micro-timescale and how these dynamics influence long-term development in externalizing behaviors. Implications for intervention studies are discussed.’
 - **Answer (GPT):** The record describes an observational, longitudinal study examining associations between coercive parent-adolescent interactions (measured via videotaped observations at ages 16–17) and subsequent substance use and antisocial behavior into early adulthood. There is no indication of a randomized controlled trial or any intervention being implemented; rather, it is a correlational study using latent growth curve and path models. Moreover, the focal interactions involve adolescents aged 16–17 (already in or beyond high school onset in most systems), and there is no mention of any childhood or pre-high school intervention. Because it clearly lacks a randomized intervention delivered before high school entry, the study fails the trial design and intervention timing criteria and should be excluded.
 - **Answer (GPT - numeric):** 0
 - **Answer (Human):** 0
- Please add a comment on whether and why you agree with the GPT decision or not:*
- **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 68:

– **Title:** ‘Effects of intensive reading remediation for second and third graders and a 1-year follow-up’

– **Author:** Blachman, B. A.; Schatschneider, C.; Fletcher, J. M.; Francis, D. J.; Clonan, S. M.; Shaywitz, B. A.; Shaywitz, S. E.

– **Year:** 2004

– **Journal:** Journal of Educational Psychology

– **Abstract:** ‘Second- and 3rd-grade children with poor word-level skills were randomly assigned to 8 months of explicit instruction emphasizing the phonologic and orthographic connections in words and text-based reading or to remedial reading programs provided by the schools. At posttest, treatment children showed significantly greater gains than control children in real word and nonword reading, reading rate, passage reading, and spelling, and largely maintained gains at a 1-year follow-up. Growth curve analyses indicated significant differences in growth rate during the treatment year, but not during the follow-up year. Results indicate that research-based practices can significantly improve reading and spelling outcomes for children in remedial programs.’

– **Answer (GPT):** The study is a randomized controlled trial of an intensive reading remediation intervention for second- and third-grade children, clearly delivered and completed in primary school, so it meets the timing requirement for the intervention. It also clearly measures cognitive outcomes (reading, spelling) at posttest and at a 1-year follow-up. However, all reported outcomes are in childhood: posttest at the end of the 8-month intervention and a follow-up only one year later, when children remain in elementary school and are well below age 18 and far from completion of upper-secondary education. There is no indication of any adult follow-up or outcomes measured at or after age 18 or post-high school. Because it is clear that the follow-up is confined to childhood and does not extend into adulthood, the record fails the adulthood outcome criterion and should be excluded.

– **Answer (GPT - numeric):** 0

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 70:

- **Title:** ‘Randomized study of combined universal family and school preventive interventions: patterns of long-term effects on initiation, regular use, and weekly drunkenness’
- **Author:** Spoth, R.; Randall, G. K.; Shin, C.; Redmond, C.
- **Year:** 2005
- **Journal:** Psychology of Addictive behaviors
- **Abstract:** ‘This study reports findings on a combined family and school-based competency-training intervention from an in-school assessment 2.5 years past baseline, as a follow-up to an earlier study of substance initiation. Increased rates of observed alcohol use and an additional wave of data allowed evaluation of regular alcohol use and weekly drunkenness, with both point-in-time and growth curve analyses. Thirty-six rural schools were randomly assigned to (a) a combined family and school intervention condition, (b) a school-only condition, or (c) a control condition. The earlier significant outcome on a substance initiation index was replicated, and positive point-in-time results for weekly drunkenness were observed, but there were no statistically significant outcomes for regular alcohol use. Discussion focuses on factors relevant to the mix of significant longitudinal results within a consistent general pattern of positive intervention-control differences.’
- **Answer (GPT):** The study is a randomized trial of a combined family and school-based preventive intervention delivered through rural schools, which likely occurred before or around early secondary school, so the intervention timing is compatible with the pre-high school criterion. However, the reported outcomes are measured only 2.5 years after baseline using in-school assessments of alcohol use, regular use, and weekly drunkenness. There is no indication of any adult follow-up (age 18+ or post-high school/grade 12). All outcomes described are during schooling and clearly not in adulthood. Because the eligibility criteria require at least one follow-up in adulthood, and it is clear that all reported outcomes are in-school and short-term, this record clearly fails the adulthood outcome criterion and should be excluded.
- **Answer (GPT - numeric):** 0
- **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 71:

- **Title:** ‘Effects of a preventive intervention on adolescent substance use initiation, expectancies, and refusal intentions’
- **Author:** Trudeau, L.; Spoth, R.; Lillehoj, C.; Redmond, C.; Wickrama, K. A.
- **Year:** 2003
- **Journal:** Prevention Science
- **Abstract:** ‘This study evaluated the effects of a school-based preventive intervention (Botvin, G. J. 1996, 2000) on growth trajectories of substance initiation (alcohol, tobacco, and marijuana), expectancies, and refusal intentions. A rural midwestern sample (N=847) provided three waves of data from middle school students. Growth curve analyses demonstrated that the intervention significantly slowed the rate of increase in substance initiation and significantly slowed the rate of decrease in refusal intentions. The intervention also slowed the rate of decrease in negative outcome expectancies, although the significance level was only marginal. A multiple group comparison showed that the impact of initial levels of substance initiation on growth trajectories of refusal intentions differed between conditions, suggesting that the intervention decreased the effect of early substance initiation on the rate of change over time for refusal intentions. Gender differences also were found, although the intervention was effective in slowing the rate of increase in initiation for both genders.’
- **Answer (GPT):** The study evaluates a school-based preventive intervention delivered to middle school students and examines growth trajectories of substance initiation, expectancies, and refusal intentions across three waves during middle school. All reported outcomes are measured in adolescence while participants are still in middle school. There is no indication of any follow-up into adulthood (age 18+ or post-high school). Therefore, the record clearly fails the adult outcome criterion and should be excluded.
- **Answer (GPT - numeric):** 0
- **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 73:

- **Title:** ‘Long-term protective factor outcomes of evidence-based interventions implemented by community teams through a community–university partnership’
- **Author:** Redmond, C.; Spoth, R. L.; Shin, C.; Schainker, L. M.; Greenberg, M. T.; Feinberg, M.

- **Year:** 2009
- **Journal:** The Journal of Primary Prevention
- **Abstract:** ‘It is becoming increasingly common for community teams or coalitions to implement programming for children and families designed to promote positive youth development and prevent adolescent problem behaviors. However, there has been only limited rigorous study of the effectiveness of community teams programming efforts to produce positive outcomes. This study employed a community-level randomized control design to examine protective parent and youth skills outcomes of evidence-based preventive interventions selected from a menu and delivered by community teams supported by a community-university partnership model called PROSPER. Twenty-eight rural communities in two states were randomized across intervention and control conditions. Data were collected through written questionnaires that were completed by approximately 12,000 middle school students in the fall of the 6th grade, prior to intervention delivery, and again in the spring of the 7th, 8th, and 9th grades. Positive intervention effects were found for youth, parent, and family outcomes (e.g., association with antisocial peers, child management, parent-child affective quality) at each post-intervention assessment point. Improvements in these family and youth skill outcomes are expected to support long-term reductions of adolescent problem behaviors, such as substance abuse. EDITORS STRATEGIC IMPLICATIONS: In this important and well controlled trial, the authors demonstrate that university partnership-supported community teams, especially when supported with ongoing technical assistance, can continue to produce positive outcomes even after much of the control over delivery of programs is turned over to representatives of the communities in which they are implemented.’
- **Answer (GPT):** The study examines the PROSPER community–university partnership using a community-level randomized controlled design, with 28 rural communities randomized to intervention or control. The interventions are preventive programs delivered to middle school students, with data collection from fall of 6th grade (baseline) through spring of 7th, 8th, and 9th grades. Outcomes include youth, parent, and family protective factors (e.g., association with antisocial peers, child management, parent–child affective quality). Although the intervention is clearly pre–high school and involves social-emotional and behavioral outcomes, all reported follow-up assessments occur during middle school and early high school years. There is no indication of any outcomes measured in adulthood (age 18+ or post–high school). Because the abstract clearly restricts follow-up to 9th grade and does not suggest any adult follow-up, the record clearly fails the adulthood outcome criterion and should be excluded.
- **Answer (GPT - numeric):** 0
- **Answer (Human):** 0
- Please add a comment on whether and why you agree with the GPT decision or not:*
- **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 74:

– **Title:** ‘Preventing substance misuse through community–university partnerships: Randomized controlled trial outcomes 4½ years past baseline’

– **Author:** Spoth, R.; Redmond, C.; Clair, S.; Shin, C.; Greenberg, M.; Feinberg, M.

– **Year:** 2011

– **Journal:** American Journal of Preventive Medicine

– **Abstract:** ‘Background Substance misuse by adolescents and related health issues constitute a major public health problem. Community-based partnership models designed for sustained, quality implementation of proven preventive interventions have been recommended to address this problem. There is very limited longitudinal study of such models. Purpose To examine the long-term findings from an RCT of a community-university partnership model designed to prevent substance misuse and related problems. Design/setting/participants A cohort sequential design included 28 public school districts in rural towns and small cities in Iowa and Pennsylvania that were randomly assigned to community-university partnership or usual-programming conditions. At baseline, 11,960 students participated, across two consecutive cohorts. Data were collected from 2002 to 2008. Intervention Partnerships supported community teams that implemented universal, evidence-based interventions selected from a menu. The selected family-focused intervention was implemented with 6th-grade students and their families; school-based interventions were implemented during the 7th grade. Observations demonstrated intervention implementation fidelity. Main outcome measures Outcome measures were lifetime, past-month, and past-year use of a range of substances, as well as indices of gateway and illicit substance use; they were administered at baseline and follow-ups, extending to 4.5 years later. Results Intent-to-treat, multilevel ANCOVAs of point-in-time use at 4.5 years past baseline were conducted, with supplemental analyses of growth in use. Data were analyzed in 2009. Results showed significantly lower substance use in the intervention group for 12 of 15 point-in-time outcomes, with relative reductions of up to 51.8%. Growth trajectory analyses showed significantly slower growth in the intervention group for 14 of 15 outcomes. Conclusions Partnership-based implementation of brief universal interventions has potential for public health impact by reducing growth in substance use among youth; a multistate network of partnerships is being developed. Notably, the tested model is suitable for other types of preventive interventions.’

– **Answer (GPT):** The study is a randomized controlled trial of preventive interventions implemented in 6th and 7th grades, which is clearly before high school entry and thus satisfies the timing of intervention criterion. However, the reported outcomes are substance use behaviors (lifetime, past-month, past-year use and indices of gateway and illicit substance use) measured up to 4.5 years after baseline. Given typical U.S. schooling ages, 6th grade corresponds to approximately age 11–12, so 4.5 years later the participants would be around 15–16 years old

and still in or around 10th–11th grade, i.e., before high school completion and likely under age 18. The abstract does not indicate any follow-up into adulthood (age 18+ or post–high school), and all outcomes are clearly within adolescence. Therefore, it clearly fails the adulthood outcome criterion and should be excluded. Decision: 0 (Exclude).

– **Answer (GPT - numeric):** 0

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 77:

– **Title:** ‘Targeting children’s behavior problems in preschool classrooms: A cluster-randomized controlled trial’

– **Author:** Raver, C. C.; Jones, S. M.; Li-Grining, C.; Zhai, F.; Metzger, M. W.; Solomon, B.

– **Year:** 2009

– **Journal:** Journal of Consulting and Clinical Psychology

– **Abstract:** ‘The present study evaluated the efficacy of a multicomponent, classroom-based intervention in reducing preschoolers behavior problems. The Chicago School Readiness Project model was implemented in 35 Head Start classrooms using a clustered-randomized controlled trial design. Results indicate significant treatment effects ($d_s = 0.53-0.89$) for teacher-reported and independent observations of children s internalizing and externalizing behavior problems. Moreover, there was some evidence for the moderating role of child gender, race/ethnic group membership, and exposure to poverty-related risk, with stronger effects of intervention for some groups of children than for others. Findings contribute to a growing area of research on poverty and preventive intervention in early childhood.’

– **Answer (GPT):** The study is a cluster-randomized controlled trial of a preschool, classroom-based intervention (Chicago School Readiness Project) targeting preschoolers’ behavior problems. The intervention clearly occurs before high school and measures social-emotional outcomes (internalizing and externalizing behavior problems) at the end of the intervention, satisfying the first two criteria. However, the abstract makes it clear that all reported outcomes are measured in preschool and there is no indication of any follow-up or outcomes assessed in adulthood (age 18+ or post–high school). The focus is exclusively on immediate post-intervention effects in early childhood, with no suggestion that the record concerns adult follow-up data. Therefore, based on the available information, the record clearly fails the adult-outcomes criterion and should be excluded.

– **Answer (GPT - numeric):** 0

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 80:

– **Title:** ‘Do charter schools improve student achievement?’

– **Author:** Clark, M. A.; Gleason, P. M.; Clark Tuttle, C.; Silverberg, M. K.

– **Year:** 2015

– **Journal:** Educational Evaluation and Policy Analysis

– **Abstract:** ‘This article presents findings from a lottery-based study of the impacts of a broad set of 33 charter middle schools across 13 states on student achievement. To estimate charter school impacts, we compare test score outcomes of students admitted to these schools through the randomized admissions lotteries with outcomes of applicants who were not admitted. We find that impacts varied considerably across schools and students, with more positive impacts for more disadvantaged schools and students and more negative impacts for the more advantaged. On average across the schools in the study, the impacts of charter middle schools on student achievement were negative but not statistically significant, regardless of whether we examined the impact of the offer of admission or actual attendance at these schools.’

– **Answer (GPT):** The study is a lottery-based (randomized) evaluation of charter middle schools, with outcomes focused on student achievement test scores during middle school. There is no indication of any follow-up into adulthood (age 18+ or post-high school) or any adult outcomes. Because the available information clearly shows that all measured outcomes occur during middle school and not in adulthood, the record fails the adult follow-up criterion and should be excluded.

– **Answer (GPT - numeric):** 0

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 86:

- **Title:** ‘KIPP Middle Schools: Impacts on Achievement and Other Outcomes’
- **Author:** Tuttle, C. C.; Gill, B.; Gleason, P.; Knechtel, V.; Nichols-Barrer, I.; Resch, A.
- **Year:** 2013
- **Journal:** Final Report. Mathematica
- **Abstract:** ‘This report shows that Knowledge Is Power Program (KIPP) middle schools have significant and substantial positive impacts on student achievement in four core academic subjects: reading, math, science, and social studies. One of the report’s analyses confirms the positive impacts using a rigorous randomized experimental analysis that relies on the schools’ admissions lotteries to identify comparison students, thereby accounting for students’ prior achievement, as well as factors such as student and parent motivation. The latest findings from Mathematica’s multiyear study of KIPP middle schools, the report is the most rigorous large-scale evaluation of KIPP charter schools to date, covering 43 KIPP middle schools in 13 states and the District of Columbia. Student outcomes examined included state test results in reading and math, test scores in science and social studies, results on a nationally normed assessment that includes measures of higher-order thinking, and behaviors reported by students and parents.’
- **Answer (GPT):** The study is a randomized evaluation of KIPP middle schools using admissions lotteries, so it satisfies the randomized trial and pre-high-school intervention criteria. However, all measured outcomes described in the abstract are during middle school: state test results in reading and math, test scores in science and social studies, a nationally normed assessment, and student/parent-reported behaviors. There is no indication of any follow-up into adulthood (age 18+ or post-high school), nor any suggestion that this report concerns adult outcomes or long-term follow-up. Because the available information clearly indicates that all reported outcomes are assessed during middle school and not in adulthood, the record fails the adult-outcomes criterion and should be excluded.
- **Answer (GPT - numeric):** 0
- **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 87:

- **Title:** ‘Pathways between alcohol use and internalizing symptoms across emerging adulthood: Examination of gender differences in interpersonal and intrapersonal processes’
- **Author:** Danzo, S.; Connell, A. M.; Stormshak, E.
- **Year:** 2021
- **Journal:** Emerging Adulthood
- **Abstract:** ‘Pathways between alcohol use and internalizing symptoms, controlling for comorbid externalizing symptoms, within a sample of 593 ethnically diverse emerging adults (age 20–22; 49% female) collected between 2013 and 2018 in the United States were examined. Peer deviance and emotional disengagement were used to examine mediation between internalizing symptoms and alcohol use. Results demonstrate that alcohol and internalizing pathways varied across gender. For males, no significant direct paths between internalizing symptoms and alcohol use were observed. However, there was an indirect path between age 20 alcohol use and reduced age 22 internalizing symptoms, via age 21 peer deviance. For females, significant bidirectional links between alcohol use and internalizing symptoms were observed from age 21 to 22. Additionally, an indirect path between age 20 internalizing symptoms and increased age 22 alcohol use via age 21 emotional disengagement was observed. Results suggest unique targets for intervention for males and females.’
- **Answer (GPT):** The study examines pathways between alcohol use and internalizing symptoms among emerging adults aged 20–22. There is no indication of any childhood or pre-high school intervention, no randomized controlled trial design, and no mention of intervention impacts on cognitive or social-emotional skills at the end of an intervention. The study appears to be an observational, longitudinal analysis of naturally occurring behaviors and symptoms in adulthood only. Therefore, it clearly does not meet the inclusion criteria for randomized childhood interventions with adult follow-up outcomes.
- **Answer (GPT - numeric):** 0
- **Answer (Human):** 0
- Please add a comment on whether and why you agree with the GPT decision or not:*
- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 88:

- **Title:** ‘Opening the “Black Box”: Family Check-Up intervention effects on self-regulation that prevents growth in problem behavior and substance use’
- **Author:** Fosco, G. M.; Frank, J. L.; Stormshak, E. A.; Dishion, T. J.

- **Year:** 2013
- **Journal:** Journal of School Psychology
- **Abstract:** ‘Family-school interventions are a well-established method for preventing and remediating behavior problems in at-risk youth, yet the mechanisms of change underlying their effectiveness are often overlooked or poorly understood. The Family Check-Up (FCU), a school-based, family-centered intervention, has been consistently associated with reductions in youth antisocial behavior, deviant peer group affiliation, and substance use. The purpose of this study was to explore proximal changes in student-level behavior that accounts for links between implementation of the FCU and changes in youth problem behavior. Data were drawn from a randomized controlled trial study of the efficacy of the FCU among 593 ethnically diverse middle school students followed longitudinally from 6th through 8th grades. Latent growth curve analyses revealed that random assignment to the FCU intervention condition was related to increased mean levels of students self-regulation from 6th to 7th grades, which in turn reduced the risk for growth in antisocial behavior, involvement with deviant peers, and alcohol, tobacco, and marijuana use through the 8th grade. Overall, these findings highlight the robust implications of self-regulation as a proximal target for family-centered interventions.’
- **Answer (GPT):** The study is a randomized controlled trial of the Family Check-Up intervention delivered to middle school students (6th–8th grades), which satisfies the requirement that the intervention occurs before high school entry. It also measures social-emotional and behavioral outcomes (self-regulation, antisocial behavior, deviant peer involvement, and substance use) during middle school, satisfying the criterion of cognitive or social-emotional outcomes at or near the end of the intervention. However, all reported follow-up outcomes are measured only from 6th through 8th grades, clearly before high school completion and before age 18. There is no indication of any adult (18+ or post-high school) outcomes. Because the available information clearly shows that no adult outcomes were measured or analyzed, the record fails the adulthood outcome criterion and should therefore be excluded.
- **Answer (GPT - numeric):** 0
- **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 89:

- **Title:** ‘Preventing adolescent depression with the family check-up: Examining family conflict as a mechanism of change’

- **Author:** Fosco, G. M.; Van Ryzin, M. J.; Connell, A. M.; Stormshak, E. A.
- **Year:** 2016
- **Journal:** Journal of Family Psychology
- **Abstract:** ‘Family-centered prevention programs are understudied for their effects on adolescent depression, despite considerable evidence that supports their effectiveness for preventing escalation in youth problem behavior and substance use. This study was conducted with 2 overarching goals: (a) replicate previous work that has implicated the Family Check-Up (FCU), a multilevel, gated intervention model embedded in public middle schools, as an effective strategy for preventing growth in adolescent depressive symptoms and (b) test whether changes in family conflict may be an explanatory mechanism for the long-term, protective effects of the FCU with respect to adolescent depression. This trial was conducted with 593 ethnically diverse families who were randomized to intervention (offered the FCU) or middle school as usual. Complier average causal effect (CACE) analysis revealed that engagers in the FCU evidenced less growth in depressive symptoms and family conflict from 6th through 9th grade, and post hoc analyses indicated that the FCU is related to lower rates of major depressive disorder. The second set of analyses examined family conflict as a mechanism of change for families who participated in the FCU. Families who reported short-term intervention benefits had significantly less escalation in family conflict over the middle school years; in turn, growth in family conflict explained risk for adolescent depressive symptoms.’
- **Answer (GPT):** The study is a randomized trial of the Family Check-Up intervention delivered in public middle schools, so the intervention occurs before high school and is eligible on that dimension. It measures adolescent depressive symptoms and family conflict from 6th through 9th grade—these are cognitive/social-emotional outcomes at and after the intervention, which also satisfies the second criterion. However, all reported outcomes are measured during middle school and up to 9th grade, i.e., clearly before age 18 and before completion of high school/upper-secondary education. There is no indication of any outcomes assessed in adulthood or beyond grade 12. Because the available information clearly indicates that no adult outcomes were measured, the record fails the adulthood outcome criterion and should be excluded.
- **Answer (GPT - numeric):** 0
- **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 91:

- **Title:** ‘A randomized trial of parents who care: Effects on key outcomes at 24-month follow-up’
- **Author:** Haggerty, K. P.; Skinner, M. L.; MacKenzie, E. P.; Catalano, R. F.
- **Year:** 2007
- **Journal:** Prevention Science
- **Abstract:** ‘This study was designed to test the efficacy of Parents Who Care (PWC), a seven-session universal prevention program which includes parenting, youth, and family components designed to prevent substance use and other problem behaviors. Using an intent-to-treat experimental design, this study tests the program efficacy across race within a balanced sample of European American (EA) and African American (AA) youth and their parents (n = 331 n AA = 163; n EA = 168). Families were recruited, randomly assigned to three conditions (group-administered [PA], self-administered with telephone support [SA], and no-treatment control) and the intervention was administered when the adolescents were in the eighth grade. Analyses on key teen outcomes of the Parent s Who Care program at 24-month follow-up are reported here and include perceptions of drug use harm; favorable attitudes about drug use; delinquent and violent behavior; and initiation into cigarette, alcohol, other drug use, or sexual activity. Repeated measures mixed model regressions found no effect of the intervention on rate of change in attitudes about drug use or frequency of delinquent or violent behavior. Regression analyses with multiple imputations for missing data detected group differences in means at 24-month follow-up. Both program formats reduced favorable attitudes toward drug use among youth (SA d = 0.39, PA d = 0.22); and AA youth in the self-administered intervention reported significantly less violent behavior than their control counterparts (d = 0.45). No effects were found for drug use harm or delinquency. Finally, logistic regression predicting a combined outcome measure of initiation of alcohol, tobacco, drug use, and/or sexual activity found AA youth in both the group- and self-administered intervention conditions significantly less likely to initiate substance use and/or sexual activity than those in the control condition. Odds ratios indicated the chances of initiating sex or substance use were reduced by almost 70% (OR = 0.31) for AA teens in the SA condition compared to controls, and 75% (OR = 0.25) for the AA teens in the PA compared to controls.’
- **Answer (GPT):** The study is a randomized trial of the Parents Who Care (PWC) universal prevention program delivered when youth were in eighth grade (middle school), so the intervention timing is eligible. However, all reported outcomes are measured at a 24-month follow-up during adolescence (approximately grades 9–10) and focus on perceptions and behaviors related to substance use, delinquency, violence, and sexual initiation. There is no indication of any outcomes measured in adulthood (age 18+ or post-high school/grade 12). Because the available information clearly shows that follow-up was limited to adolescence, the record does not meet the adulthood outcome criterion and should be excluded.
- **Answer (GPT - numeric):** 0
- **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

- **Model:** gpt-5.1-2025-11-13
- **Run date:** 2026-09-10

STUDY-ID: 92:

- **Title:** ‘Staying Connected with your Teen and the promise of self-directed prevention programs’
- **Author:** Haggerty, K. P.; Klima, T.; Skinner, M. L.; Catalano, R. F.; Barkan, S.
- **Year:** 2015
- **Journal:** In Family-Based Prevention Programs for Children and Adolescents (pp. 221-240). Psychology Press
- **Abstract:** ‘Problem behaviors in adolescence and young adulthood, such as substance use, violence, and risky sexual practices, carry heavy social, economic, and health consequences for individuals and entire communities (Catalano et al., 2012; Rehm et al., 2009). Research in developmental and prevention sciences (Hawkins, Catalano, & Miller, 1992) has identified key antecedents, or risk factors, for these behaviors. For instance, favorable parental attitudes towards antisocial behavior, lack of clear guidelines for behavior, poor monitoring, harsh or inconsistent discipline, and high levels of family conflict are predictive of more adolescent problem behavior. In adolescence, more proximal risk factors include peer norms that problem behavior is acceptable, perceptions of little harm as a result of engaging in these behaviors, and association with deviant peers (Catalano, Haggerty, Hawkins, & Elgin, 2011; Hawkins et al., 1992; Herrenkohl, Lee, & Hawkins, 2012). Research has also identified protective factors that lead youth toward prosocial paths and away from problem behaviors. Examples of protective factors include strong parent-child and school bonding, opportunities for active involvement in the family and other institutions (e.g., school), and recognition for positive behaviors (Herrenkohl et al., 2012).’
- **Answer (GPT):** The record is a conceptual/overview article discussing risk and protective factors for adolescent and young adult problem behaviors and the promise of self-directed prevention programs. The abstract provides no indication that it reports original quantitative results from a randomized controlled trial. There is no description of random assignment, no sample, no intervention evaluation design, and no report of outcomes, either at the end of an intervention or in adulthood. Furthermore, the focus is on adolescence and young adulthood (“teen”), implying that any discussed interventions would not clearly be restricted to ending before high school entry, as required by the inclusion criteria. Given that it appears to be a narrative or theoretical piece rather than a trial report or follow-up, and it clearly lacks evidence of being an RCT with measured outcomes, it should be excluded.

– **Answer (GPT - numeric):** 0

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10

STUDY-ID: 93:

– **Title:** ‘Long-term effects of the Strong African American Families Program on Youths’ Conduct Problems’

– **Author:** Brody, G. H.; Kogan, S. M.; Chen, Y.; McBride Murry, V.

– **Year:** 2008

– **Journal:** Journal of Adolescent Health

– **Abstract:** ‘Purpose The Strong African American Families program, a universal intervention to deter alcohol use among rural African American preadolescents, was evaluated to determine whether it also prevented conduct problems across the 29 months separating the pretest and long-term follow-up assessments. The program is based on a contextual model in which intervention effects on parental behavior and youth protective factors are hypothesized to lead to behavior changes. Methods African American 11-year-olds (N = 667) and their primary caregivers were randomly selected from public school lists of fifth-grade students and randomly assigned to an intervention (n = 369) or control (n = 298) condition. Intervention families participated in a 7-week family skills training program designed to deter alcohol use. Each meeting included separate, concurrent sessions for parents and children, followed by a joint parent-child session during which the families practiced the skills they learned in their separate sessions. Control families were mailed leaflets regarding early adolescent development, stress management, and exercise. All families completed in-home pretest, posttest, and long-term follow-up interviews during which parent-report and self-report data regarding conduct problems, low self-control, deviance-prone peer affiliations, parenting, and youth protective processes were gathered. Results Intent-to-treat analyses indicated that prevention-group youth were less involved than control-group youth in conduct problems across time. As hypothesized, prevention effects were stronger for youth at greater risk of developing conduct problems. Intervention targeted parenting and youth factors partially accounted for intervention effects among high risk youth. Conclusions Although the Strong African American Families program was designed to deter underage drinking, it is also effective in preventing the development of conduct problems.’

– **Answer (GPT):** The study evaluates the Strong African American Families program with African American 11-year-olds (fifth graders), randomly assigned to intervention or control, so

it is a randomized trial of a pre-high-school intervention and includes relevant social-emotional outcomes (conduct problems, self-control, deviance-prone peer affiliations, parenting, youth protective processes) at posttest and a 29-month follow-up. However, all follow-up assessments occur during early adolescence (approximately up to age 13–14) and clearly not in adulthood (age 18+ or post-high school). There is no indication of any adult follow-up or outcomes. Because it clearly lacks adult outcome measurement, the record fails the adulthood criterion and should be excluded.

– **Answer (GPT - numeric):** 0

– **Answer (Human):** 0

Please add a comment on whether and why you agree with the GPT decision or not:

– **Model:** gpt-5.1-2025-11-13

– **Run date:** 2026-09-10